3

The transformation of charlatanism, or the moral treatment

A request to the office of the minister of the interior in 1810 reveals the importance, in Napoleonic bureaucratic circles, of a particular book. The prefect of the department of the Meurthe wrote:

> There exists in my Department, about three kilometers from the departmental capital, an institution called Maréville devoted exclusively to the reception of the insane (*les insensés*). I have already had occasion to communicate with your Ministry about its administration and the condition of its buildings. . . . The regimen of this interesting establishment has attracted my special concern, and I have recognized that, while the inmates would be considered very well treated if Maréville were only a simple hospice, the establishment leaves something to be desired with respect to its special aim – which is to succeed, while it is still possible, in restoring the mental faculties of the insane. . . . I thought, Sir, that . . . the book which indicates the method for the treatment of the insane followed by Doctor *Pinel* would provide me with trustworthy information. . . . I beg you to be so kind as to have the book sent to me. I will hasten to give an account of the arrangements that my reading of it leads me to adopt.[1]

At about the same time, a former cavalry officer in the Napoleonic army, enrolled at the state veterinary school at Alfort for a course in "rural economy" to help him prepare for an administrative career, decided to "instruct myself on the administration of public institutions" by visiting those in the vicinity. Among them was an establishment for the insane about which he prepared an extensive memoir, evaluating it in terms of the criteria set forth in "the treatise on insanity of Monsieur Pinel."[2] Nor was the appeal of the book confined to those with medical-bureaucratic interests. The young novelist-in-the-making Stendhal, always eager for a theoretical framework to guide his native insights into the human psyche, presented himself at the Paris Medical School one morning in 1805 "in order to read the *Aliénation mentale* of Pinel"; a year later, trying

[1] AN: F^{15} 2603, letter of October 18, 1810; *Pinel* underlined in the original.

[2] Archives de Paris et de l'ancien département de la Seine: D3AZ 212, "Notice sur l'établissement consacré au traitement de l'aliénation mentale établi à Charenton, près Paris," by H. de Colins, dated June 16, 1812, ms. pp. 1–5.

to acquire the book for his personal library, he was informed that the first and only printing was "entirely sold out."[3]

The book in question was Dr. Philippe Pinel's *Traité médico-philosophique sur l'aliénation mentale, ou la manie,* first published in 1801 and with a second, expanded edition appearing in 1809. It represented the most up-to-date and complete account of its subject to appear in France; and unlike the only text with which it can be meaningfully compared, the 1785 *Instruction* of Colombier and Doublet, it did not merely collect and codify accepted medical wisdom on the treatment of insanity but recommended a fundamental innovation: the "moral treatment" (*traitement moral*). Cursorily defined – the definition will be fleshed out in the course of this chapter – the moral treatment meant the use for the cure of insanity of methods that engaged or operated directly upon the intellect and emotions, as opposed to the traditional methods of bleedings and purgings applied directly to the lunatic's body. While it did not entail a total abandonment of the old repertory of physical remedies, it did entail an acknowledgment of their grave insufficiency.

Pinel, it must be stressed, never claimed to be the absolute creator of the moral treatment. From his earliest published references to it, which predate the Revolution, he acknowledged the derivative nature of the concept. He routinely assigned its distant origins to the ancients, citing the precepts of Celsus and Caelius Aurelanius; and he invariably noted its use among English practitioners of his own day, advising that they be emulated on that account. Particularly conspicuous among Pinel's Englishmen was Francis Willis, who during the period 1788–89 assumed the weighty responsibility of treating King George III's madness and who thus brought *moyens moraux* to widespread public attention.[4] One of Pinel's disciples added to this genealogy Samuel Richardson's novel *Sir Charles Grandison* (1753–54; French translation 1755–56), arguing that it had done much to propagate the idea of the moral treatment among "all classes of society."[5] When Richardson's fictional Ital-

[3] The quotes are from Stendhal's journal of 1805, cited in Jules C. Alciatore, "Stendhal et Pinel," *Modern Philology* 45 (1947): 118; and from a letter he received in 1806, cited in V. Del Litto, *La vie intellectuelle de Stendhal* (Paris: Presses universitaires de France, 1959), p. 289 n76.

[4] Pinel, "Observations sur le régime moral qui est le plus propre à rétablir dans certains cas la raison égarée des maniaques," *Gazette de santé,* 1789, no. 4: 13–15. The article mentions Willis. English parliamentary committees summoned Willis and others who treated George III during his bout with insanity to testify on "the state of His Majesty's health"; these parliamentary depositions were often reprinted in cheap popular editions – hence the particularly widespread knowledge of Willis's methods. See Richard Hunter and Ida Macalpine, eds., *Three Hundred Years of Psychiatry, 1535–1860* (London: Oxford University Press, 1963), p. 509.

[5] Moreau de la Sarthe, "Médecine mentale," *Encyclopédie méthodique,* Series: Medicine, Vol. 9 (1816), p. 138.

ian aristocrat Clementina succumbs to madness and is bled with leeches to no avail by local medical men, two English physicians "eminent for their knowledge of disorders of the head" are summoned, with the rationale that "the English physicians [are] more skillful than those of any other country in the management of persons afflicted with such maladies"; and "partly in pursuance of [their] advice," a new and ultimately successful course of "sooth[ing] and humour[ing] the Lady Clementina" is embarked upon.[6] In short, the proposition that insanity might require "moral" rather than physical treatment had begun to gain currency in the second half of the eighteenth century, and Pinel's *lack* of priority in the matter was never disputed. What Pinel did claim for himself was that he was the first to explicate the moral treatment fully: One combed the English literature in vain, he said, for a detailed description of the technique sufficient to guide the novice; the English "rivals" perversely touted it while keeping it, for all practical purposes, "impenetrably veiled."[7] He also claimed that he was the first to place the technique on a scientific footing.

Pinel's rendition of the moral treatment functioned as the originative psychiatric paradigm[8] – that is, as the puzzle-solution around which a community of like-minded medical men specializing in insanity first formed in France. It was the crucial additional element enabling the foundations of psychiatry discussed in the previous chapter to be built upon, focusing and energizing the whole venture. At least as early as 1806, Pinel began to recognize this development, then still in its infancy; he alluded to the existence of a new specialty, calling it "the medicine which has for its object the lesions of the functions of the understanding."[9] And in 1805 one of his students, in a dissertation on the causes and cure of insanity, also acknowledged the emergent spe-

[6] Samuel Richardson, *The History of Sir Charles Grandison*, ed. Jocelyn Harris (London: Oxford University Press, 1972), Vol. 2, pp. 189–91, 193–94, 313, 480. Richardson was familiar with the English medical literature of his day concerning madness; see the editor's note to *Grandison*, Vol. 2, p. 673.

[7] Pinel, "Observations sur le régime moral," p. 13; "Recherches et observations sur le traitement moral," (henceforth R & O), *M.S.M.E.* 2 (Year VII): 215–55, esp. 216.

[8] A word about the loose and selective nature of my borrowing from Kuhn is in order. While I use the term "paradigm" throughout this book to refer to the moral treatment and its key role in the formation of the French psychiatric specialty, I have set aside the full Kuhnian model of scientific change in which that term figures. The ambiguities lodged in the term, as Kuhn employed it, are well suited to my purposes. A paradigm is both theory and technique; and even more important here, its meaning emphasizes the interpenetration of text and context. As a cognitive text, a paradigm offers a worked-out solution to a particular scientific problem; as a social-normative context, it provides a basis for the common loyalty and activity of the professional community that organizes around it. I could find no satisfactory synonym for the multilevel and evocative "paradigm."

[9] See his letter of 11 nivôse Year XIV in E. Szapiro, "Pinel et Esquirol: Quelques commentaires sur les débuts d'une amitié" *Ann. m.-p.* 134 (June 1976): 61.

cialty, making more explicit its connection to its paradigm. Many men, he said, recoiled from the extensive contact with lunatics required by Pinel's moral treatment; and thus they revealed themselves as temperamentally unsuited to pursue the medical study of madness, "to cultivate this branch of the healing art."[10] In light of the founding role played by the moral treatment and the shaping influence it continued to exert, it is essential to understand the precise content of the paradigm, how it was derived, and how it was legitimated. A selective narrative of the career of the founder himself will open the way.

Philippe Pinel: a medical career in political context

Born in a small town in Languedoc in 1745, the son and nephew of physicians, Philippe Pinel received a degree from the faculty of medicine in Toulouse. But he was disappointed with the quality of his medical education – the "knowledge required for promotion to the doctorate was scant," he later recalled – and to satisfy his intellectual appetite spent an additional four years studying at the Faculty of Medicine of Montpellier. As he became increasingly absorbed in such avant-garde lines of inquiry as the applications of mathematics and the physical sciences to medicine and the methodological standards that those more advanced sciences set for his chosen field, he felt himself called to Paris; for, he remembered musing, "could one follow the progress [of medicine] anywhere but in the capital, where instruction in it is so superior?"[11]

The intellectually ambitious but personally shy provincial arrived in Paris in 1778, equipped only with a Toulouse medical degree which had no validity in his new place of residence. He soon met with distressing rebuffs at the hands of the Paris Faculty of Medicine: He twice failed in the competition for the prix de Diest, a scholarship for poor students which would have enabled him to complete free of charge the requisite training at the Paris faculty for medical certification in the capital and admission into the Regency. In the second of these competitions, held in 1784, the jury's verdict stressed Pinel's "painful" mediocrity in all areas of medical knowledge – an assessment so incompatible with everything that is known about Pinel's intellectual accomplishments that it is impossible not to suspect an ulterior motive. The men of the faculty may well have been punishing Pinel for his friendship with two prominent members of the Royal Society of Medicine – Thouret and the persona

[10] E. Esquirol, *Des passions considerées comme causes, symptômes et moyens curatifs de l'aliénation mentale* (Paris: Thèse de médecine, Year XIV/1805), p. 7.

[11] See Pinel's autobiographical reminiscences in "Analyse appliquée à la médecine," *Dict. des sci. med.*, Vol. 2, pp. 26–27.

non grata Fourcroy. In any case, Pinel found his experience with the faculty so profoundly depressing that he contemplated emigration to America.[12] His earliest first-hand impressions of the Paris medical community – "I see in those who exercise this respectable profession [in Paris] only baseness and intrigues," he wrote home to his brother[13] – had been amply confirmed.

Pinel's disillusionment with official medicine in the capital city where he had expected so much from it – a disillusionment which focused on its elitist and exclusive professional structure – is comparable to the experience of other members of his generational cohort. The aspiring young philosophes who came to the capital from the provinces in the 1770s and 1780s, for example, found that the much-vaunted "republic of letters" was a closed shop; and they responded to this discovery by becoming bitter Grub Street writers whose pre-Revolutionary productions were radically populist in outlook.[14] For his part, Pinel embraced a kind of medical populism, although as will be discussed later, he prudently gave expression to it only after the outbreak of the Revolution.

Employment as a medical journalist – in 1784 he was made editor of the *Gazette de santé*, a four-page weekly not quite of Grub Street calibre but many notches below the prestigious *Journal de médecine* – enabled Pinel to earn a satisfactory living and ended his ruminations about quitting his native country.[15] At about the same time, he began to develop an intense interest in the study of mental illness. The incentive was a personal one: A friend had developed a "nervous melancholy" that had "degenerated into mania." Pinel must have strongly identified with this young man, fourteen years his junior, whom he described as a provincial "of a gentle and timid character" who had "settled in Paris in order to devote himself to science." The friend easily completed the usual course of legal study and was admitted to the bar, but like Pinel, he could not rest with ordinary professional achievement: "The desire to distinguish himself and to acquire knowledge prompted him to seek out . . . the most efficacious means of making rapid progress." Under the sway of this ambition, he became eccentric, withdrawing into solitude and adopting a vegetarian diet in the belief that meat eating dulled the brain. Pinel stood by helplessly – "I was reduced to the role of spectator" – as his friend's adherence to this austere regimen became

[12] P. Chabbert, "Philippe Pinel à Paris (jusqu'à sa nomination à Bicêtre)," *Comptes rendus du XIXe Congrès international de l'histoire de la médecine* (Basel and New York: S. Karger, 1966), pp. 590–92.

[13] Letter of December 8, 1778, in Casimir Pinel, "Lettres de Pinel précédées d'une notice plus étendue sur sa vie," *Gaz. hebd. de med.* (June 17, 1859): 374.

[14] On this group, see Robert Darnton, "The High Enlightenment and the Low-Life of Literature in Pre-Revolutionary France," *Past and Present* (May 1971): 81–115.

[15] Chabbert, "Pinel à Paris," p. 592.

unshakable, his initial enthusiasm about his brilliant professional prospects turned into despair, and his agitation increased. Eventually there was nothing for Pinel to do but bring him to the lunatic ward of the Hôtel-Dieu, where a treatment consisting of baths and restorative nourishment seemed to calm him. But before the treatment was completed, the lawyer's anxious parents insisted that he return home to his village, where, immediately falling into despondency, he fled his surveillants and wandered into the woods. Some days later he was found dead, Plato's dialogue on the immortality of the soul poignantly clutched in his hands.[16]

The tragedy – and the gross mismanagement of the ailment, which suggested that the tragedy was unnecessary – seems to have haunted Pinel. It led him to seek employment at one of the best-known private sanatoria in Paris for the treatment of insanity, the *maison de santé* on the rue de Charonne, owned by Belhomme, a former cabinetmaker. Pinel became the physician of the establishment, a technically irregular status because he had no Parisian medical credentials. Despite his contempt for the mercenary Belhomme (whom he described as having a "marked indifference . . . to the cure of rich patients, or rather an unequivocal desire to see the remedies [employed on them] fail"),[17] he remained there for the five years preceding the Revolution, gathering observations on insanity and beginning to formulate his views on its nature and treatment.[18]

When the Revolution broke out, Pinel was eminently well situated to reap professional benefit from it. In addition to his ties with members of the Royal Society of Medicine, he was a friend of Cabanis, under whose sponsorship he had become a habitué of the "enlightened" salon of Madame Helvétius at Auteuil and hence a member of the circle of intellectuals later known as the Idéologues. His pro-Revolutionary sentiments were patent: In 1790 he became an *officier municipal* of Paris; in 1795 he hailed the birth of his first son in a personal letter by calling him "a little republican."[19] With the coming of the Revolution and the insistence that the "medical constitution" of France be overhauled, the Paris Faculty of Medicine, always unreceptive to Pinel, declined in

[16] The case is recounted in "Observation sur une melancolie nerveuse dégénerée en manie," *Gazette de santé*, 1786, no. 9: 34–35.

[17] R & O, pp. 218–19.

[18] Chabbert, "Pinel à Paris," p. 594. Pinel was invited to present a preliminary paper on this subject before the Royal Society of Medicine in 1788, and it was, he recalled, "very well received," though never published. R & O, p. 218 n1.

[19] Chabbert, "Pinel à Paris," p. 590. Pinel's letter to his brother dated November 7, 1792, in *Gaz. hebd. de med.* (September 24, 1858): 669, recounts his political activity of 1789, also noting that he was temperamentally ill-suited to the *tourbillon* of political life and withdrew after a year. The birth announcement is in a letter to his brother dated 26 messidor Year III quoted in René Semelaigne, *Aliénistes et philanthropes: Les Pinel et les Tuke* (Paris: Steinheil, 1912), p. 218.

influence, and decision-making power in medical affairs passed to "new men," often affiliated with the Royal Society of Medicine – and often Pinel's friends. Thouret became an advisor to the legislative Comité de mendicité appointed in 1790 to make recommendations about public institutions for the indigent and infirm; a new commission to administer the Paris hospitals, appointed by the municipality in 1791 after the resignation of the directors of the old Hôtel-Dieu and *hôpital-général*, included Thouret and Cabanis; and when the opening of the Ecole de médecine de Paris to replace the defunct faculty was decreed in 1794, Fourcroy headed the committee in charge of finding personnel, and Thouret emerged as the permanent dean.[20]

Some of these "new men" had already served as physician–bureaucrats under the Old Regime. Furthermore, the Maison du Roi, active in health affairs under the Old Regime, survived the Revolution, being rechristened the Ministry of the Interior in 1790 and still numbering hospitals and public assistance among its responsibilities.[21] Thus it is hardly surprising that the bureaucratic concern of the 1780s about rehabilitative institutions for the insane was carried directly into the revolutionary period. The very first of the many visits to public institutions made by the Comité de mendicité was to Bicêtre (Thouret was present); and when the Comité came to draw up its recommendations, they included, in the specializing spirit of Tenon, the "installation of two hospitals devoted to the cure of madness, until now treated only at the Hôtel-Dieu." In one of these projected hospitals, the committee advised additional treatment for those who had previously been labeled "incurable" and consigned to the *hôpital-général*.[22] The Paris municipality, surveying its public institutions at the same time as the national government was doing so, recorded testimony from the administrators of Bicêtre that also suggested the desirability of taking therapeutic initiatives with the allegedly incurable insane. No, the mayor's lieutenant for hospitals was told, there was no "curative method adopted for madness." "All the madmen sent to Bicêtre remain there *in statu quo* until it pleases nature to favor them." Still, about one-fifth of these inmates "annually recover their good sense and are returned to their families," and about three-fourths of these spontaneous cures came from the group which had undergone prior, unsuccessful treatment at the Hôtel-Dieu.[23]

[20] D. B. Weiner, "Le droit de l'homme à la santé – une belle idée devant l'Assemblée constituante, 1790–91," *Clio medica* 5 (1970): 211; Jean Imbert, *Le droit hospitalier de la Révolution et de l'Empire* (Paris: Sirey, 1954), p. 35 and p. 35 n111; and David M. Vess, *Medical Revolution in France, 1789–1796* (Gainesville: University Presses of Florida, 1975), pp. 169–72.

[21] See Clive H. Church, *Revolution and Red Tape: The French Ministerial Bureaucracy, 1770–1850* (Oxford: Clarendon Press, 1981), pp. 54–55, 157, 159.

[22] Alexandre Tuetey, ed., *L'assistance publique à Paris pendant la Révolution: Documents inédits*, Vol. 1 (Paris: Impr. nationale, 1895), pp. ii–iii, xv.

[23] Ibid., p. 237.

The outcome of these overlapping investigations was the decision by the new Paris hospital commission (which included Thouret and Cabanis) to "medicalize" the section for the insane at Bicêtre by appointing a physician-in-chief for the entire institution.[24] In 1793 the commission chose Pinel, who with his prior experience at the *maison* Belhomme (which he had recently brought to public notice in an article in the medical journal edited by Fourcroy),[25] his Idéologue views, and his ties to Thouret and Cabanis, was clearly the man for the job.[26] Shortly thereafter, a second professional plum fell into Pinel's lap – a chair in hygiene at the newly formed Ecole de médecine de Paris, an appointment in which Fourcroy was certainly instrumental.[27] In 1795 Pinel was transferred from Bicêtre to its counterpart for women, the Salpêtrière. In 1802 the Salpêtrière, still primarily a depot for the inveterately insane, received an infusion of curable *folles* from the Hôtel-Dieu, a redistribution of population designed in part to enable Pinel, as it was announced, "to give new breadth to [his] researches . . . on insanity."[28] Later that month the Conseil général des hospices de Paris – just formed by Minister of the Interior Chaptal (Pinel's old friend from his Montpellier days) and including among its members the ubiquitous Thouret – gave additional support to the researches of "citizen Pinel." It authorized him to select two medical students from among the *externes* at the Salpêtrière to aid him in "the new study, with which he has been charged, of the madwomen of that *maison*," and to reward these assistants with extra pay.[29] The

[24] Before the Revolution, the section was an entirely nonmedical operation, directed by a lay "governor" and staffed by attendants responsible for cleaning, distributing food, and maintaining security; ibid., pp. 236–37.

[25] "Observations sur une espèce particulière de mélancolie qui conduit au suicide," *La Médecine éclairée par les sciences physiques* 1 (1791): 154–59, 199–201. Pinel bluntly referred to *Médecine éclairée* as Fourcroy's medical journal in *Traité médico-philosophique sur l'aliénation mentale, ou la manie,* 1st ed. (Paris: Richard, Caille et Ravier, Year IX/1801), p. 187n.

[26] On the role of the commission in naming Pinel to Bicêtre by decree of August 25, 1793, see Semelaigne, *Aliénistes et philanthropes,* p. 43. The historical record is riddled with contradictions about the precise date of Pinel's arrival at Bicêtre; see, e.g., Casimir Pinel, "Deux lettres de Pinel," *Gaz. hebd. de med.* (September 24, 1858): 667–68.

[27] See Vess, *Medical Revolution,* pp. 170, 172. Within a year, Pinel was moved to the chair of internal medicine, vacated by the initial appointee – none other than François Doublet, coauthor of the 1785 *Instruction.*

[28] The announcement was made to the assembled Paris Medical School; see AN: AJ[16] 6308, "Séance publique de l'Ecole de médecine de Paris du 5 brumaire an XI, Discours du citoyen Hallé," p. 25.

[29] See decree of the Conseil général des hospices, 21 brumaire Year XI, quoted in Mireille Wiriot, *L'enseignement clinique dans les hôpitaux de Paris entre 1794 et 1848* (Paris: Thèse de médecine, 1970), p. 67. On Chaptal's friendship with Pinel and the founding and membership of the Conseil général des hospices, see J.-A. Chaptal, *Mes souvenirs sur Napoléon* (Paris: Plon Nourrit, 1892), pp. 18–20, 62–63.

Revolution had indeed, as Pinel once gracefully understated it, "opened for me a freer career."[30]

Pinel and the "concierges": the origins of the moral treatment

In the late eighteenth century, medical writers occasionally acknowledged – or perhaps let slip – the virtual indistinguishability of medical therapeutics and quackery in the area of insanity: Charlatans[31] could be as effective as fully accredited physicians, if not more so, in treating difficult cases of madness. According to the *Encyclopédie* article, "Mania is one of those diseases with which the most skillful physicians ordinarily fail, while the charlatans, the persons with secrets (*gens à secret*), very often succeed." And, the Encyclopedist continues, "Felix Plater recounts having seen a quack (*empyrique*) who cured all maniacs by bleeding them up to seventy times a week. A horde of celebrated physicians (*praticiens*) assure us that they know of no remedy for mania as efficacious as that." Similarly, arguing for the use of hellebore on maniacs whose condition had proved resistant to other purgatives, the *Instruction* of Colombier and Doublet noted, "Some happy examples of the application of this remedy in desperate cases by [the] learned physician [Lorry], several other equally favorable attempts by means of this medicament in the hands of charlatans, are authentic and powerful facts which ought to persuade us to have recourse to it."[32] With Pinel, recognition of the efficacy of charlatanistic treatments of insanity not only continued but was taken further: Selected aspects of charlatanism were to be deliberately appropriated by official medicine and transformed by it – transformed rather more in status than in content.

In the introduction to the first edition of his *Traité*, Pinel explained that his key concept, the moral treatment, derived from charlatanistic practice. "Men who [are] strangers to the principles of medicine, guided only by sound judgment or some obscure tradition, have devoted themselves to the treatment of the insane, and they have effected a great many cures." These men were "concierges," superintendents of the insane whose caretaking function was diffused over the

[30] R & O, p. 219.

[31] Here, as elsewhere in this book, I am using the term "charlatan" as it was used by contemporaries. Although many historians of medicine now make an analytic distinction between the charlatan, who deliberately dupes the public for personal gain, and the respectable lay healer, no such strict distinction was observed in eighteenth-century French usage. In the writings of that period, anyone who practiced medicine without the requisite university degree stood to be called a *charlatan* or *empirique* (quack), terms which were used interchangeably. See the *Encyclopédie* article "Charlatan" (Vol. 7, p. 326) for explicit comment on this point of usage.

[32] "Manie," *Encyclopédie*, Vol. 20, pp. 958–59; and [Jean Colombier and François Doublet], *Instruction sur la manière de gouverner les insensés* . . . (Paris: Impr. royale, 1785), p. 28.

entire day and night, who had "the habit of living constantly in the midst of lunatics." Pinel's roster of them, gleaned from the recent literature and his own personal experience, was international in scope and included some of the Englishmen whom he had long considered versed in the moral treatment.[33] The famous Francis Willis, caretaker to regal madness (he had treated the Queen of Portugal as well as King George III), was first among them. The proprietor of a private madhouse in Lincolnshire since 1776, Willis was a cleric who had perfunctorily acquired a medical degree when harassed by the authorities to regularize his practice among his parishioners; his certification in both divinity and medicine earned him the nickname "the Duplicate Doctor." Pinel's source, however, described Willis only as "that respectable ecclesiastic" and stressed his continued activity in a pastoral capacity. Since Willis never in fact presented himself as a member of the medical community or as a possessor of scientific expertise, Pinel's labeling of him as a layman is essentially correct even if technically inaccurate.[34] Also listed by Pinel were John Haslam, "the apothecary of the Bethlehem Hospital ['Bedlam'] in London"; the "honest concierge" of the public establishment for the insane in Amsterdam, who had held the job for thirty years and had recently been written up in the Idéologue journal *Décade philosophique;* Père Pouction, an old monk who tended the insane in an establishment in the French Alps; and most important, Pussin, the "governor" of the insane at Bicêtre, whom Pinel met and worked with after his appointment to that hospice in 1793. A tanner by trade, Pussin had as a young man in the 1770s been successfully treated for scrofula at Bicêtre; and, following a familiar pattern – for large public institutions are worlds unto themselves – the former inmate was eventually recruited onto the staff of the hospice.[35] Elsewhere Pinel indicated that Madame Pussin, who aided her husband in his daily work, was also gifted in the moral treatment.[36]

Why should learned medicine look for guidance to such a motley crew? Pinel was quite clear about his reasons for advocating this unlikely strategy. The first was a matter of medical epistemology. During the course of the eighteenth century, as belief in deductive medical theoretical systems was replaced by reliance upon clinical observation – a shift given an official imprimatur in 1794 when the Ecole de médecine was established with Fourcroy's pedagogical dictum "Read little, see and do

[33] Introduction to *Traité,* 1st ed., pp. xliii–xlv; p. xliv n1–5; p. xlv n1.

[34] Ida Macalpine and Richard Hunter, *George III and the Mad-Business* (New York: Pantheon, 1969), pp. 269–70; "Détails sur l'établissement du docteur Willis pour la guérison des aliénés," *Bibliothèque britannique,* Series: Literature (1796): 760, 763–4, cited by Pinel in *Traité,* 1st ed., p. xliv n1.

[35] On Pussin's biography, see Semelaigne, *Aliénistes et philanthropes,* p. 501, which quotes the registers of Bicêtre on Pussin's 1771 admission.

[36] R & O, p. 247.

much"[37] – the "empiricism" that had become a synonym for quackery no longer seemed an entirely bad thing. Rather, it could have great redeeming scientific value; for as Pinel noted about the concierges, "the continual spectacle of all the phenomena of insanity" necessarily supplied them with a "multifarious and detailed knowledge that is lacking in the physician," whose contact with insane patients was "most often limited to . . . transitory visits."[38] By virtue of the structure of their often humble jobs, the lay concierges had simply acquired more information about insanity than bona fide medical men possessed. They were specialists in the old sense: lay healers who narrowed their focus to one ailment and treated it repetitively.

Pinel's second reason for gravitating to the concierges was a matter of political ideology. Why had the skills of these men not been recognized earlier? The oversight was, of course, the result of the elitism of the medical corporations. "An eternal struggle seems to have been set up, from the very first centuries that medicine was in existence, between a blind quackery (*empirisme*) and the legal and regulated exercise of medicine, between those who, through their limited knowledge or their appetite for lucre, develop an exclusive favor for certain remedies, and a class of men who submit themselves under the authority of the laws to preliminary courses of study, to tests of ability and knowledge." Which side to take? Given his loaded description of the conflict, Pinel seems to be leaning in the predictable direction ("the choice is doubtless easy"), but in medias res he suddenly turns around, an awkward textual maneuver that inadvertently discloses the disjunction between his real emotions and what he knew to be the "correct" opinion on the matter. "Public opinion," he observes, does not always choose the official practitioner; it vacillates, often favoring the quack because of his manifest successes as a healer or because he is the underdog in the corporate system, the "victim of a sort of tyrannical oppression" with whom the public identifies and to whom it rallies by a "natural interest." The public is not, Pinel now admits, wrong in its preferences. "What intolerance! What insulting contempt has often been lavished on men some of whom have talents, others the precious results of a long experience . . ." Warming to his new theme, Pinel declares that he would have medicine operate, like other sciences, as a noncorporate career open to talent and as a free marketplace of ideas:

> I very much wish that in medicine a sound judgment, a natural wisdom, an inventive mind, *devoid of all other privilege*, counted for something – the way they do in physics, in chemistry, in botany; that people cared little if

[37] Fourcroy, "Rapport et projet de décret sur l'établissement d'une école centrale de santé à Paris," 7 frimaire Year III, in A. de Beauchamp, ed., *Enquêtes et documents relatifs à l'enseignement supérieur*, Vol. 28 (Paris: Impr. nationale, 1888), p. 204.

[38] Introduction to *Traité*, 1st ed., pp. xlv–xlvi.

> a particular man had completed a certain course of study or fulfilled certain formalities, but only if he had examined thoroughly some aspect of medical science or if he had discovered some useful truth.

During his two years at Bicêtre, Pinel had, he said, become acutely aware that this kind of respect for the unaccredited was necessary if "some progress in the doctrine of insanity" was to be made. An invigorating new perspective was required; Pinel needed "to exit from a certain circumscribed circle" of traditional writings and hackneyed modes of seeing. And the obvious way to do this was to "enrich the medical doctrine of insanity with all the knowledge acquired by a sort of quackery (*empirisme*)" – that of the concierges.[39]

A defiant medical populism, then, accompanied Pinel's inauguration of the moral treatment. He sounded the note again in the closing paragraph of his introduction, thus greatly amplifying it:

> A work of Medicine published in France at the end of the eighteenth century ought to have a different character from one written in an earlier epoch: a certain soaring flight in the ideas, a wise freedom, and especially the spirit of order and research that reigns in all parts of natural history ought to distinguish it: no longer should egoistic (*particulier*) views or *the interest of a powerful corporation* dictate it.

In an article of 1798 that served as a trial run for one of the chapters of the *Traité*, Pinel had made the same point, holding up for opprobrium the physician "endowed with an exclusive confidence in his own knowledge and full of a doctoral pomposity (*bouffisure doctorale*)." French medicine, he told his readers, had just been "released from the fetters [i.e., its corporate structure] that gave it its spirit of routine . . . and its disfavor in public opinion."[40] Pinel's avowed populism was rooted, as a form of belated revenge, in his own pre-Revolutionary experience; and while it was particularly well-suited to the Revolutionary period of de jure medical deregulation in which he wrote, Pinel was even then rare among official medical men in espousing it.[41] That this populism was, on both sides of the Atlantic, the characteristic late eighteenth-century translation of republican political values into the medical idiom can be seen in the views of the eminent Philadelphia physician Benjamin Rush. In a lecture of 1789 on the practice of medicine as explicitly accommodated to "the history of the American revo-

[39] Ibid., pp. xlii–xliii, xlvi–xliv, my italics.

[40] Ibid., p. lvi, my italics; and Pinel, "Mémoire sur la manie périodique ou intermittente," *M.S.M.E.* 1 (1798): 94–119, esp. 118.

[41] Clearly, though, Pinel tailored the expression of his medical populism to the historical moment. When he reprinted the "Introduction to the First Edition" in the 2d edition of the *Traité* (Paris: Brosson, 1809) – at the height of the Imperial regime and after medical regulation had been reimposed – he carefully purged it of extreme populist comments, condensing the discussion of the concierges and changing the *corporation puissante* of the last paragraph to the innocuous *imagination ardente* (p. xxxii).

lution," Rush spoke with the accents of the ebullient Pinel of the period of the First French Republic. "A formal and pompous manner" – Pinel's *bouffisure doctorale* – "whether accompanied by a wig, a cane or a ring, should avoided" by the physician. "Improvement in medicine" would come not only from the requisite colleges and universities, where "men of genius and learning" produced "systems of physic," but also from ordinary people "in every walk of life." "Remember how many of our most useful remedies have been discovered by quacks. Do not be afraid, therefore, of conversing with them. . . . But further. In the pursuit of medical knowledge, let me advise you to converse with nurses and old women. . . . Even negroes and Indians have sometimes stumbled upon discoveries in medicine. Be not ashamed to inquire into them."[42]

Pinel's contemporaries took note of the immediately populist origins of the moral treatment and expressed neither surprise nor dismay. Cabanis observed that Pinel's ideas on the subject had come to him from "the ingenious and respectable Pussin, superintendent of madmen." The Idéologue economist Jean-Baptiste Say attributed the cure of a large number of lunatics at Bicêtre to the *moyens purement moraux* of Citizen Pussin. And, in keeping with his general outlook, Pinel remained open-minded toward other varieties of "charlatanism" as well. Although mesmerism, or animal magnetism, had been under official medical censure in France since 1784, Pinel is reported to have cordially received one of its chief exponents, the Marquis de Puységur, at the Salpêtrière in 1812. He told his visitor that he had read his works "with much interest"; he watched Puységur magnetize the young patient he had brought with him and, while prudently refusing to pronounce for or against "the new curative procedures you apply" for lack of sufficient evidence, he invited the layman to return that winter "in order to try out on some of [the madwomen of the Salpêtrière] the power of magnetic influence."[43]

Lest Pinel seem to be a thoroughgoing advocate of medical ecumenism or even of the laissez-faire model of a medical profession, it must be stressed that the founder of French psychiatry tempered these

[42] Benjamin Rush, "Observations on the Duties of a Physician and the Methods of Improving Medicine Accommodated to the Present State of Society and Manners in the United States" (1789), printed as a separately paginated appendix to his *Medical Inquiries and Observations* (Philadelphia: Prichard & Hall, 1789), pp. 21–39, esp. pp. 24–25, 36. I am grateful to Lamar Riley for calling this text to my attention.

[43] Cabanis, *Rapports du physique et du moral de l'homme, O.P.* Vol. 1, 587 n1 (Cabanis's own note); J.-B. Say, "Des hôpitaux et des hospices de Paris," *Décade philosophique* (20 floréal Year IX): 262; A.-M.-J. de Chastenet de Puységur, *Les fous, les insensés, les maniaques et les frénétiques ne seraient-ils que des somnambules désordonnés?* (Paris: J.-G. Dentu, 1812), pp. 81–83. On Puységur, see Henri F. Ellenberger, *The Discovery of the Unconscious: The History and Evolution of Dynamic Psychiatry* (New York: Basic Books, 1970), pp. 70–74.

democratic attitudes with a more conventional elitism. The wisdom of the concierges was, to his mind, radically incomplete. "Lacking the most rudimentary knowledge of the development of the human understanding, could [the concierges] put order and precision into their observations or even elevate themselves to a language appropriate for rendering their ideas?" In order that quackery be incorporated into medicine it had first to be "brought back to those general principles of which it is destitute."[44] It had, in other words, to be philosophized.

Like Cabanis, the Idéologue Pinel was insistent upon a philosophically grounded medicine. And he frequently noted the indispensable philosophical foundations of a medicine of insanity in particular: "Will one be able to delineate all the alterations and perversions of the functions of the human understanding if he has not deeply contemplated the writings of Locke and Condillac and familiarized himself with their principles?"[45] Thus the lay concierge, as diligent, perceptive, and talented as he might be, was inalterably the intellectual inferior of the *médecin-philosophe*. The latter would take the rough-hewn commonsensical knowledge of the former and transform it into something refined, scientific, and esoteric; the elite professional confraternity, at one moment threatened with dissolution by Pinel, was thus fundamentally – and quickly – restored by him. Pinel's ambivalent perch between populism and elitism was revealed in his candid account of his relationship with Pussin at Bicêtre in the years 1793–95:

> The dogmatic tone of the doctor was abandoned from that point on [i.e., from the moment that Pinel realized that Pussin had a great deal to teach him]. Frequent visits, sometimes during several hours of the day, helped me to familiarize myself with the deviations, vociferations and extravagances of the most violent maniacs. From that point on, I had repeated conversations with the man best acquainted with their anterior condition and delirious ideas: utmost attention to humor all the pretensions of his amour-propre, questions that were varied and went back over the same material when the answers were obscure, no objections on my part when what he advanced was doubtful or improbable, but a tacit return to further examination in order to illuminate or rectify it.[46]

In this way, Pinel used Pussin as a resource person, grateful to the layman for his wealth of information but always aware that this tanner-turned-healer was no equal; Pinel's manner, as he himself ingenuously reports it, is entirely condescending. Elsewhere – in the same text in which he criticized "doctoral pomposity" – Pinel summed up the via media between populism and elitism that characterized his attitude toward the practice of medicine. "Rousseau, in a fit of caustic humor,

[44] Introduction to *Traité*, 1st ed., pp. xlvi, xlix.

[45] *Traité*, 1st ed., p. 45 (also "Mémoire sur la manie périodique" p. 119); virtually the same sentence appears in *Traité*, 2d ed., Introduction, p. xi.

[46] *Traité*, 1st ed., Introduction, p. xlviii.

once called for Medicine and told her to come without the physician: he would have better served humanity by raising his eloquent voice against conceited ineptitude and by summoning talent and genius to the study of [this most necessary] science."[47]

Pinel's turn to charlatanism and to the wisdom of the laity to obtain material for medical-scientific theory must be seen in another context as well. It is analogous to the initiative taken by Cabanis in his *Rapports du physique et du moral* to incorporate into scientific medicine, on the theoretical grounds of psychophysiological reciprocity, the commonsensical concern for the emotional state of the sick individual, the precepts of soothing, reassuring, and of sustaining hope – the bedside manner, or what in the parlance of the day was frequently called a "medicine of the imagination." For Cabanis such a seemingly populist strategy had the thoroughly professional-imperialist aim of helping to constitute medicine as the omnibus science of man. Pinel shared the Idéologues' grand view of medicine, as an impromptu comment to his brother, written a few months after the republican revolution of 1792, indicates. "It must be admitted that, in the present disorganization of almost all the structures (*états*) of society, the profession of medicine will play one of the most glorious roles because it really exists *in* nature and it is from physicians that one can recover the greatest amount of accumulated knowledge."[48] For Pinel, as for Cabanis, the scope of medical knowledge was so uniquely vast because medicine embraced the domains of *le moral* and *le physique* and the relations that obtained between them, because it included a "medicine of the imagination."

Pinel's interest in a "medicine of the imagination" (to which he and Cabanis often referred under the more sober name of "moral hygiene")[49] is abundantly evident in his early writings. In one of his pre-Revolutionary *Gazette de santé* editorials, for example, he discoursed on the effects that an "exalted imagination" could wreak on the body. "The causes of [some people's] fears, often chimerical in the eyes of a man of sangfroid, can produce in them extreme upsets and real diseases." Thus a young ecclesiastic, who had playfully consulted a fortune teller and a reader of horoscopes, was told by both that he would die at the age of twenty-five. This double prediction provoked "the most violent alarm in his soul," and before too long "fever seized him, his body became withered and dessicated, and all the remedies lavished upon him during the course of a year were useless." A complete cure, however, was brought about by the uneventful passage of his

[47] Ibid., p. 45 (also "Mémoire sur la manie périodique," p. 119).

[48] "Lettres de Pinel," *Gaz. hebd. de med.* (September 23, 1859): 599, letter of November 16, 1792.

[49] Cabanis regarded the moral treatment as part of *hygiène morale; Rapports, O.P.*, Vol. 1, p. 587. See also his *Coup d'oeil sur les révolutions et sur la reforme de la médecine, O.P.*, Vol. 2, pp. 221–25.

twenty-fifth birthday. And commenting in the *Gazette* on a case of impotence in a young scholar, Pinel's prognosis was optimistic. It was "the diverse sentiments that besiege his soul" which had "suspended, as by enchantment, the functions of the organ of generation." Montaigne's "excellent essay on 'Imagination' " would, said Pinel, corroborate this view and confirm Pinel's own counsel: that time, patience, and the natural process of growing accustomed to the nuptial bed would clear up this apparently physical affliction. Similarly, in his entry in the 1793 prize essay contest held by the Society of Medicine on "the best manner of teaching practical medicine in a hospital setting," Pinel included a section on the *remèdes moraux* to be employed with patients of all types. During the hospital visits that were part of his own medical education, he said, he had become "convinced of the happy effects produced on patients by consoling and reassuring words" delivered at their bedside; and he proposed that this approach be stressed in the training of young physicians. Medical students in the wards were to be instructed to apply themselves to "reviving the patients' flagging courage and dispelling forebodings of doom from their imaginations."[50]

The moral treatment of the insane, then, was a subset of the *remèdes moraux* recommended by both Cabanis and Pinel to aid the recovery of patients suffering from any and all diseases. Both men were committed to taking these *remèdes moraux* from "charlatanistic" practice and assimilating them to official medicine by means of a theoretical or philosophical gloss. Odd as its provenance may seem, the moral treatment was really in the mainstream of the most advanced Idéologue medicine. Indeed, Pinel's work in helping to constitute medicine as the all-embracing "anthropology" through his championing of the moral treatment can be seen quite clearly if we examine for comparative purposes a text of 1787 which Pinel himself certainly read (for he cited it in the *Gazette de santé* two years later): a traveler's report about the treatment of the insane at "Bedlam" in London, followed by recommendations for their treatment at Bicêtre and the Salpêtrière. The author of the latter, the Abbé Robin, was the king's chaplain, and he came out strongly in favor of *moyens moraux* as the necessary accompaniment of baths and bleedings. "The art of healing madmen," he asserts, "is still in its infancy, and in order to possess it, it would be necessary to

[50] Pinel, "Hygiène: Les accès de mélancolie ne sont-ils pas toujours plus fréquens et plus à craindre durant les premiers mois de l'hiver?" *Gazette de santé* (1787): 201–2; "Extrait d'un mémoire à consulter sur une impuissance provenante d'une cause morale. Réponse des auteurs de la *Gazette de santé*," ibid. (1786): 18–80; and *Mémoire sur cette question proposée pour sujet d'un prix par la Société de médecine . . .* , in *The Clinical Training of Doctors: An Essay of 1793*, ed. Dora B. Weiner (Baltimore: Johns Hopkins University Press, 1980). I have translated from pp. 47–48 of the French text of this bilingual edition.

extend one's quests beyond the limits of medicine: it would be necessary to study *le moral* of patients sometimes even more than *le physique*."[51] While Pinel would have essentially concurred in this therapeutic prescription, he would have disagreed violently about the "limits" Robin placed on the proper domain of medicine. For at the core of Pinel's position was the insistence that medicine be defined so as to comprehend rather than exclude *le moral*.[52]

What was the moral treatment?

Pinel, it will be remembered, regarded as one of his two major contributions with respect to the moral treatment that he, unlike his secretive English predecessors, had specified the method fully. A review of the first edition of the *Traité* by the Idéologue physician Richerand took Pinel at his word and praised him precisely for supplying this much-needed explication. But not everyone, even among Pinel's most intimate and supportive circle of colleagues, proved so obliging. Cabanis, though willing to grant Pinel's claim that the moral treatment was genuinely curative, at least "for a rather large number of cases," expressed disappointment at the vagueness of his friend's use of the concept. Esquirol, the most important student and collaborator of Pinel, called Pinel's conceptualization the "fruit of observation and genius," but at the same time admitted that no thorough definition of the moral treatment had really been offered. The best approach both to understanding it and to demonstrating its efficacy was, he suggested, through an examination of cases.[53] This was, in fact, Pinel's own basic mode of presentation in the *Traité*. "Little stories (*historiettes*)," he once noted, when they were the "true results of observation," were serious scientific business.[54] Such remarks translate readily into Kuhnian terms. The paradigm of the moral treatment was not so much succinctly expressed in "shared rules" as embedded in an "arsenal of

[51] Robin, *Du traitement des insensés dans l'hôpital de Bethléem de Londres, traduit de l'anglois, suivi d'observations sur les insensés de Bicêtre et de la Salpêtrière* (Amsterdam, 1787), pp. 46–51, 55. Pinel's reference to Robin in "Observations sur le régime moral" was solely for the purpose of corroborating his own point about the intolerable superiority of the "rival" English nation in the area of treatment of insanity.

[52] See, for further confirmation of Pinel's views on this matter, *Traité*, 1st ed., Section 6, which, under the heading "Principles of the Medical Treatment of Lunatics," includes *both* traditional physical treatments and case histories of treatment by "moral" means.

[53] A. Richerand, review of Pinel, *Traité*, in *Journal de médecine* 1 (nivôse an IX): 366; Cabanis, *Rapports, O.P.*, Vol. 1, p. 587; Esquirol, *Des passions*, pp. 7, 13, 78. On Richerand as Idéologue physician, see Martin S. Staum, *Cabanis: Enlightenment and Medical Philosophy in the French Revolution* (Princeton, N.J.: Princeton University Press, 1980), pp. 251–53.

[54] *Traité*, 1st ed., pp. 228, 231–32.

exemplars," of "shared examples of successful practice."[55] Hence any attempt to clarify the nature of that therapy must begin with a look at these exemplars.

The exemplars are plentiful, even if we confine ourselves to the first edition of the *Traité,* which presents, scattered through the text and in varying degrees of detail, about a dozen cases.[56] From this group, I have selected four "full-length" cases and a pair of very condensed and elliptical ones. All are cases for which Pinel provided a certain amount of commentary and explication. All were singled out in an informed contemporary reading of the text – the combined review-abridgment of the first edition of the *Traité* prepared by Pinel's disciple Moreau de la Sarthe for the Idéologue journal *Décade philosophique.*[57] All the patients described in the cases are men: The sample comes from Pinel's years at Bicêtre; only after his transfer to the Salpêtrière did he systematically apply "moral methods" to women. I will recount the cases as narratives, adhering closely to Pinel's own choice of words, and will then gather and systematize his own middle-level generalizations about the therapeutic principles they illustrate. In the next section, I will examine the same material from another perspective by taking up the broader theoretical justifications of the moral treatment and higher-level generalizations about its fundamental principles.

Case 1: the furious soldier.[58] A soldier was brought to Bicêtre after the standard treatment for insanity at the Hôtel-Dieu failed to produce an improvement in his condition. He suddenly became dominated by the single idea of his imminent departure for the army. The "ways of gentleness" (*voies de la douceur*) were tried on him in vain, and force had to be applied to get him into his cell; since he tore everything to shreds during the night, he also had to be tied up. He spent a week in this condition, relentlessly violent, hurling invectives at Pinel. Then one morning during Pinel's rounds, his manner changed. In a submissive tone, humbly kissing the chief physician's hand, he said to Pinel, "You promised to let me at liberty inside the hospice if I were calm. Well, I am calling upon you to keep your word." Pinel smiled with pleasure and spoke to him kindly, at that same moment releasing him

[55] See "Postscript–1969" to Thomas S. Kuhn, *The Structure of Scientific Revolutions,* 2d ed. (Chicago: University of Chicago Press, 1970) and esp. "Second Thoughts on Paradigms" in Kuhn, *The Essential Tension: Selected Studies in Scientific Tradition and Change* (Chicago: University of Chicago Press, 1977), pp. 307, 318–19.

[56] Other major sources are Esquirol's *Des passions,* presenting cases in which Pinel and Pussin often figure, and his earlier article "Observations sur l'application du traitement moral à la manie," *Journal général de médecine* 1 (Year XI): 281–94.

[57] See Jacq. L. Moreau de la Sarthe, "Art de guérir: Traité médico-philosophique sur l'aliénation mentale par Ph. Pinel," *Décade philosophique* (20 prairial Year XI): 458–67.

[58] *Traité,* 1st ed., pp. 58–59 (also R & O, pp. 223–24).

from all physical constraints, for these were now, he believed, superfluous or even harmful. Seven months later the soldier left the hospice, completely cured; he never experienced a relapse.

Case 2: the fasting Catholic.[59] A young man, dismayed by the reverses Catholicism had suffered in France during the Revolution, became a maniac and ended up in Bicêtre after an unsuccessful treatment at the Hôtel-Dieu. He was filled with a "dark misanthropy," spoke only of the torments of hell, and thought that he could avoid them by imitating the abstinence and mortifications of the ancient anchorites; hence he steadfastly refused to take any nourishment. After four days of this fast, Pussin suddenly appeared at his door with a group of hospital attendants, all armed and carrying chains which they rattled noisily. Pussin addressed the patient with fiery eyes and a thundering voice, saying that unless he ate during that night the soup they had brought him, he would incur the cruelest punishments. The patient did eventually eat the soup and, after this turning point in his conduct, submitted to a diet that restored his physical strength; gradually the use of his reason was restored as well.

Case 3: the clockmaker.[60] One of the most famous clockmakers in Paris succumbed to mania through a confluence of circumstances: his own obsession with building a perpetual motion machine, which kept him sleepless, working through the night; and "the recurrent terrors incited [in him] by the storms of the Revolution." His loss of reason took a singular form. He believed that he had been decapitated by the guillotine, that his head had been mixed in pell-mell with those of other victims, and that when the judges belatedly reversed their verdict, he had been given back a head other than his own – and a far less beautiful head at that. After a period of violent furor alternating with delirious gaiety, he calmed down somewhat. And because he remained obsessed with the perpetual motion machine, drawing sketches of it all over the walls of the hospice, Pussin allowed him to set up a sort of workshop in the antechamber of his cell furnished with clockmakers' tools supplied by his relatives. He worked with undivided attention for a month, and thought that he had succeeded; but just as he was hailing himself as the new Archimedes, the mechanism stopped. To save face before his fellow inmates and Pussin, the clockmaker declared that he could easily fix the machine but that he had grown tired of the whole enterprise and did not wish to bother. Thus the obsession with the perpetual motion machine was spontaneously laid to rest.

To "combat and destroy" the delirious idea about the alleged change

[59] Ibid., pp. 59–61 (also R & O, pp. 224–25).
[60] Ibid., pp. 66–70 (also R & O, pp. 229–31).

of heads, the complicity of a convalescent lunatic at Bicêtre was enlisted. This jovial man began to discuss with the clockmaker the miracle of Saint Denis, who carried his head in his hands and covered it with kisses as he walked. The clockmaker strongly defended the possibility of such an event, citing his own decapitation as confirmation. His partner now burst out laughing and said in a derisive tone: "Madman that you are, how do you think Saint Denis would have been able to kiss his head; was it with his posterior?"[61] This unexpected reply struck the clockmaker sharply and he retreated in confusion from the mocking laughter. He no longer spoke of his alleged mishap at the guillotine, and serious work at his clockmaker's occupation over the next several months fortified (*raffermir*) his reason. He was returned to his family and never had a relapse.

Case 4: the guilt-ridden tailor.[62] During the Jacobin phase of the Revolution, a tailor happened to muse in public on the sentence of Louis XVI. From that time on, his patriotism became suspect in his section. Exaggerating the significance of some menacing words that he overheard one day and believed directed at himself, he came home trembling, unable to eat or sleep, continually fearful. After treatment at the Hôtel-Dieu he was brought to Bicêtre; the idea that he had been condemned to death on the guillotine now absorbed him entirely. Pinel arranged for him to resume his occupation, to receive a small salary for repairing the clothing of the other inmates of the hospice. The tailor threw himself into this work; "nothing equaled his zeal for making himself useful," and after about two months he ceased to mention his alleged death sentence and seemed virtually cured. But several months later, his old melancholy returned. Pinel was at this time leaving his post at Bicêtre for one at the Salpêtrière, but he devised a treatment plan for the tailor and put Pussin in charge of executing it. Three young physicians dressed in the black suits of magistrates presented themselves at the hospice as a commission of the Corps législatif delegated to carry out the trial of the tailor. They interrogated him about his conduct, his favorite newspapers, his political opinions, and "the one with the most grave and imposing demeanor" then loudly proclaimed the verdict: acquittal. "We acknowledge having found in him only the sentiments of the purest patriotism." The "trial" relieved the tailor of his symptoms – although the cure, Pinel admitted, proved only temporary.

Case 5: some victims of stifled ambition.[63] A man in a high position falls into disgrace and shortly thereafter into a deep melancholy – that is

[61] R & O gives *derrière; Traité,* 1st ed. substitutes the more discreet *talon* or heel.
[62] *Traité,* 1st ed., pp. 233–37.
[63] Ibid., pp. 238–39.

what the physician Erasistratus once ingeniously called a "stifled ambition" (*ambition rentrée*). A comparable case is presented by the soldier who led the successful assault on the Bastille and later became a lunatic confined to Bicêtre. The expedient treatment for this man would not be baths and showers but rather a captain's commission. Another such case, recounted by the sixteenth-century physician Forestus, concerns a rich German merchant who experienced a commercial setback. Though minimal in itself, this reverse so deeply affected his imagination that he believed his resources totally depleted and himself condemned to die of hunger. In vain did people remind him that he still had an immense fortune; even when they spread out before him all the riches contained in his coffers, he dismissed this as a false appearance. But the Reformation was able to accomplish what the medicaments and adroit treatments of Forestus could not. The merchant became absorbed night and day in the zealous defense of popery and ended by being entirely cured of his melancholy.

The most striking feature of the treatments employed in several of these cases is their theatricality: Scenes and spectacles are staged. The patient, however, is not aware of their fictive nature or, in the instance of the joke told to the clockmaker, of their deliberate and planned nature. These treatments are, to use a phrase of the period, "pious frauds," deceptions that take advantage of the gullibility of an individual for his own benefit,[64] or, as Pinel sometimes described them, "innocent ruses."[65] The theatricality of the moral treatment is underscored textually by Pinel's frequent use of the word *appareil* – that is, display, show, pomp, appearance. The visitation to the fasting Catholic is an "*appareil* suitable to induce fright"; the make-believe commission of the Corps législatif presents itself to the melancholic tailor with the full "*appareil* of authority."[66] This kind of therapeutic theater was not new but had been traditionally used (by both medical men and charlatans) much more in the treatment of melancholy than of mania – hence Pinel's insistence that "the moral treatment *of mania* is one of the most important, and, until now, least advanced parts of the medicine of observation."[67] The *Encyclopédie* article "Melancholy," a good typical source, recommends a variety of curative "ruses," as they are called there. If, for example, a melancholic believes that he has a live animal in his belly, "one must pretend (*faire semblant*) to remove it" – to give

[64] See, e.g., the entry "Fraud: Should Pious Frauds be Practiced on the Common People?" in Voltaire, *Philosophical Dictionary*, trans. P. Gay (New York: Harcourt Brace & World, 1962), pp. 279–83, a dialogue on whether the common people need fables in order to be persuaded of the existence of God.

[65] *Traité*, 1st, ed., p. 95 (also R & O, p. 248).

[66] Ibid., pp. 60, 235 (also R & O, p. 225).

[67] Ibid., p. 103, my italics (also R & O, p. 254).

the patient a strong purgative and then "to throw this animal adroitly into the basin when the patient is not looking."[68] Theatricality is, however, an instrument of the moral treatment rather than one of its principles; its therapeutic significance will emerge when we enumerate the principles underlying the five cases.

The first is the overriding preference given to gentleness (*douceur*) and "the ways of gentleness" (*voies de douceur*), recurring terms in Pinel's vocabulary. This commitment to "philanthropy"[69] was not just an abstract ethical principle but also a practical therapeutic one, not just an end in itself but a means to an end. For the lunatic was not to be considered "absolutely deprived of reason," inert and "inaccessible to motives of fear and hope, to sentiments of honor."[70] He was, rather, feelingful and responsive; and hence *douceur* – in the form of "consoling words, the happy expedient (*artifice*) of reviving the hope of the lunatic and gaining his confidence" – produced therapeutic success, as "daily experience attests," while "maltreatment, or the ways of a too harsh repression, exacerbates the illness and can render it incurable."[71] The young Esquirol recounted in his dissertation a successful case in which this consoling *douceur,* and the doctor–patient rapport and camaraderie that it produced, was the central motif. A young man had suffered two brief bouts of insanity and consulted Pinel when he perceived in himself all the warning signs of a third. Entrusted with the case, Esquirol seems to have devoted most of his energy to cheering on the patient, who, "tormented by the fear of an imminent relapse," struggled to prevent its occurrence. On the brink of the patient's descent into madness, we find Esquirol "incessantly encouraging [him] with consoling words," using "forceful language in order to rouse his battered courage." The crisis passes, with a tranquil look coming over the patient's face and a light sweat spreading over his body. "You have saved me," he tells Esquirol, and that same evening the two play a game of billiards together.[72]

Unhappily, *douceur* is by itself often an inadequate principle: It can be ignored or exploited by the patient, as the case of the furious soldier

[68] "Mélancolie," *Encyclopédie,* Vol. 21, pp. 419–20. (It is unclear from the text whether or not the healer was obliged to produce a real animal.) There was, in other words, a more or less continuous tradition in the use of "moral means" for the treatment of melancholy, beginning with the counsels of the ancients and extending through the eighteenth century; but this medical tradition had lapsed with respect to mania. On ancient theatricality in the cure of mania, see Pinel, "Observations sur le régime moral," p. 13, on the *annonces spécieuses* of Celsus.

[69] *Traité,* 1st ed., pp. 52, 63, 86 (also R & O, pp. 219, 242).

[70] Ibid., p. 99 (also R & O, p. 251); Pinel is quoting approvingly the formulation of a Swiss-French Dr. Delarive in a 1798 article on the use of the moral treatment at the York Retreat.

[71] Ibid., p. 97 (also R & O, p. 250).

[72] Esquirol, *Des passions,* pp. 80–81.

exemplifies. Or, as Pinel wrote of a megalomaniacal patient who struck everyone he encountered and ordered them to prostrate themselves before him and do homage, "What good could come of the *voies de douceur* . . . with a lunatic who regarded other men as particles of dust?" In such cases, *douceur* must be momentarily abandoned and recourse had to the opposite principle – "an imposing *appareil* of repression." Pinel was quite explicit about the mentality that must accompany therapeutic acts of repression; they must be "exempt from feelings of animosity and anger."[73] They are to be construed as necessary evils, never to be sadistically relished but responsibly undertaken so that, as with the furious soldier, the desired mode of *douceur* can be fruitfully restored. Sometimes "it is necessary to subjugate [lunatics] first, and encourage them afterward."[74]

As regrettable as the task of repression might be, it was an integral part of the work of a healer of the insane, and his skill in this area was at a premium. The essential skill – if something so intangible can be called a skill – was to exude authority. Francis Willis was almost legendary in this regard, as the description of him cited by Pinel indicates. "On first meeting with a new patient, his usually gentle and friendly countenance changed its expression. He suddenly became a different figure, commanding the respect even of maniacs. His piercing gaze seemed to read their hearts and divine their thoughts at the very moment that they were being formed. In this way he gained control over them, which he used as a means of cure." Indeed, during parliamentary inquiries on George III's health, Edmund Burke questioned Willis's judgment in allowing the King to shave himself, doubting that Willis could have handled His Majesty had he suddenly become furious while brandishing the razor. Unruffled, Willis replied that he would have commanded him "by the EYE! I would have looked at him *thus*"; and Burke immediately averted his own eyes, graphically acknowledging Willis's rather astounding ocular ability to convey his authority.[75]

Pinel did not require that all practitioners of the moral treatment be possessed of such stunning powers, but he did stipulate that they be able somehow to communicate authority by their presence, their "physical and moral qualities."[76] What inhered naturally in their persons might, of course, be legitimately supplemented by theatrical devices – Pussin's fearsome chain-rattling band of men in the case of the fasting Catholic, for example. This theme of salutary coercive authority was a very old one in Pinel's thought on the treatment of the insane.

[73] *Traité,* 1st ed., pp. 61, 66, 101 (also R & O, pp. 225, 229, 252).
[74] Ibid., pp. 99–100, quoting Delarive (also R & O, pp. 251–52).
[75] Ibid., p. 48 (also R & O, pp. 216–17). Burke is cited in Macalpine and Hunter, *George III and the Mad-Business,* p. 272.
[76] *Traité,* 1st ed., p. 58 (also R & O, p. 223).

As early as 1786, in his analysis of the mania of his young lawyer friend, he had ascribed the futility of the situation to the fact that "no one had *enough authority over him*" to compel him to end his austere and unhealthy regimen; and in 1798 he repeated, almost obsessively, this same analysis of that personally crucial case: "In my treatment of mania [in the early 1780s], it was in my power to use a great number of remedies, but I lacked the most powerful . . . , that which consists in the art of subjugating and subduing the lunatic."[77]

The third principle of the moral treatment implicit in these cases is that of "combating and destroying the delirious idea" that characterized the insanity.[78] It was dependent on the first two principles – that is, on having previously made the patient minimally receptive to such intervention, either by securing his confidence through gentle means or by forcibly commanding his respect through repressive ones. Pinel and Pussin used two basic methods in this "idea-combat." One was distraction or diversion, turning the lunatic's attention away from the idea which, in his pathological condition, pervaded his entire being. Such distraction could be accomplished by engaging him in useful work, exemplified in the cases of the tailor and the clockmaker, or by having him listen to music.[79] The second method was to subject the lunatic to a startling or jolting experience, to "shake up the imagination" (*ébranler fortement l'imagination*), as Pinel usually expressed it,[80] in a manner calculated to discredit a particular idea. Sometimes a mild jolt sufficed, such as the joke told to the clockmaker, which caught him completely off guard and seems to have made him aware of the ludicrous illogic of his position regarding the aftermath of decapitation. Usually the jolt was more vigorous and sustained, such as the full-dress enactment of the trial of the tailor, with its official proclamation that the patient's dominant idea was erroneous and henceforth to be regarded as null and void. The theatricality of the moral treatment was particularly pronounced in the implementation of this method, for an appropriately jolting incident had to be staged, and staged dramatically.

The fourth and final principle concerns managing pathological passions as opposed to combating pathological ideas, and it reflects the distinction implicitly made by Pinel between the ideational and affective components of the moral treatment, a distinction parallel to Cabanis's definition of the two components of the domain of *le moral*. In

[77] Pinel, "Mélancolie nerveuse," p. 35, my italics; R & O, p. 223.

[78] *Traité*, 1st ed., p. 69 (also R & O, p. 231).

[79] On music, see Esquirol, *Des passions*, pp. 77–78. Some years later, at the Salpêtrière, Pinel made work a standard part of his treatment program, installing a "vast atelier of knitting and sewing" in which convalescent lunatics were employed for a small salary. *Traité*, 2d ed., pp. 372–73.

[80] See, e.g., R & O, p. 235, and *Traité*, 1st ed., pp. 76, 236.

his comments on the cases of "stifled ambition," Pinel said explicitly that diseased passions should not be destroyed outright – although, as we have seen, this was a completely suitable policy for delirious ideas – but "counterbalanced by more powerful affections" or else simply "not thwarted."[81] Hence the proposal to cure the man who led the assault on the Bastille in 1789 by giving him the formal recognition of his prowess that he craved but had been denied – that is, by satisfying his passion. The sixteenth-century German Reformation case illustrates a fortuitous operation of the strategy of counterbalancing: Anxiety over a commercial failure preoccupies the merchant entirely and cannot be mitigated by realistic considerations, but it is pushed into the background by a suddenly incited religious passion.

One of the corollaries of the principle of therapeutic management of the passions was the necessity of ascertaining just what the patient's dominant passions were. Unlike delusional ideas, which Pinel believed made their way repeatedly into the patient's conversation and tended to be flaunted, the crucial pathological passions might be deliberately concealed. This was one of the major obstacles to the study of insanity. "The extreme distrust of everything around them that marks lunatics . . . often leads them to dissimulation or taciturnity. It would be clumsy technique and a blunder (*maladresse*) to indicate to them an overt intention to observe them and to penetrate their secret thoughts with questions." Pinel instead recommended "a tone of candor, an extreme simplicity and an affectionate manner" as a way of "triumphing" over their self-protective reserve. The intuition and tactics of a detective were useful as well. Recounting how Galen figured out that the insomnia and continual agitation of a well-born lady (who evaded all direct questions about the origins of her malady) had been provoked by her amorous passion for an actor, Pinel commented that it was regrettable that Galen "did not apply himself especially to the study of insanity," for he possessed a necessary skill in that line of work, "a rare sagacity for uncovering a hidden moral affection."[82] The young Esquirol advised that the servants of wealthy insane patients be actively recruited into the treatment process and that they play the role of trusted confidant and friend, never participating in any of the repressive measures that the physician might deem necessary. In this way they could often furnish the physician with "the ideas, thoughts, projects of the lunatic . . . , a multitude of things useful for the restitution of health." This seems to have been a time-honored therapeutic motif. Galen got some of his most important information from the lady's slave, and in Richardson's *Sir Charles Grandison*, Clementina's attendant Camilla serves precisely the function that Esquirol later recom-

[81] *Traité*, 1st ed., p. 238.
[82] Ibid., Introduction, pp. xiii n1, lii, lv–lvi.

mended, of indulgence toward her melancholic mistress and communication of private information about her to family and physicians.[83]

"Scientizing" the treatment

In the account of the moral treatment given thus far, its non-esoteric, lay origins – which Pinel so proudly and defiantly proclaimed – are abundantly evident. We must turn now to Pinel's promise to add to the moral treatment something that the concierges could not supply, to upgrade it to a level suitable to a medicine with scientific pretensions, to "bring it back to the general principles of which it is destitute." There are several ways in which he attempted to make good this promise.

Connecting treatment to etiology. For the concierges, the moral treatment was a pragmatic method, devised by trial and error, applied because it worked. These laymen had no inclination to extend it beyond the bounds of its specific utility, to see what light its efficacy shed on other problems concerning insanity. Thus a memoir prepared by the concierge Pussin – and sent by Pinel to the minister of the interior in 1798 as a supporting document in the request to transfer Pussin from Bicêtre to the Salpêtrière so that he might continue his collaboration with Pinel – delineated a variety of *remèdes moraux* that had proven reliable, spoke out against the debilitating effects of bleeding on lunatics, but went no further.[84] For Pinel, however, the success of the moral treatment in certain cases was a piece of data in a larger jigsaw puzzle; it was seen as a function of the etiology of those cases – that is, the cases in question were presumed to have a "moral" cause, a cause rooted in ideas, or more especially in the passions.[85] This does not mean, of course, that the body was not implicated, for the Idéologue Pinel took as an article of faith the interrelation of *le moral* and *le physique:* Human ideas and passions had a significant impact on the "animal economy."[86] But while an insanity susceptible to cure by the moral treat-

[83] Esquirol, *Des passions,* pp. 52–3; Pinel, Introduction to *Traité* 1st ed., p. xiii n1; and Richardson, *Grandison,* Vol. 2, pp. 201–2, 297.

[84] The document, "Observations faites par le citoyen Pussin sur les fous," was discovered by Dora B. Weiner, who published the French text in "The Apprenticeship of Philippe Pinel: A New Document," *Clio Medica* 13 (1978): 125–33. Samuel Tuke, the lay director of the York Retreat, explicitly refused to theorize about the moral treatment, believing that the medical propensity for premature systematization impeded the progress of therapeutic knowledge. See Andrew T. Scull, *Museums of Madness: The Social Organization of Insanity in Nineteenth-Century England* (New York: St. Martin's Press, 1979), p. 158.

[85] *Traité,* 1st ed., title of Section 3, para. 3, p. 110.

[86] Ibid., Introduction, p. xxi.

ment affected the body functionally, it could not be the result of a true organic lesion.

Contrary to the findings of eighteenth-century anatomists, whose dissections of the cadavers of lunatics had revealed almost universal abnormality and degeneration in the brain substance, Pinel cited the evidence of dissections conducted by himself and others to the effect that insanity could exist without discernible brain lesion or cranial malconformation.[87] "According to Pinel," wrote Cabanis, "this [latter] class of insanities is much more extensive than one would have thought"; and that is precisely why Pinel counseled such heavy reliance on the moral treatment.[88] In those cases where the anatomical substratum of the mental faculties had suffered a lesion, Pinel abandoned not only confidence in the moral treatment but all his therapeutic optimism, admitting the incurability of the condition. This was his pronouncement on the famous "wild boy of Aveyron," a child who had allegedly reared himself in the forests of southwestern France and who was brought to Paris in 1800 amid great expectations on the part of savants that he could not only be taught to speak but could have all his intellectual and moral faculties awakened. Pinel examined him and found "no sign of the capacity for improvement (*perfectibilité*)"; the boy was in his opinion afflicted with idiotism caused by a congenital deficit in his cerebral constitution or its defective development in early childhood. The one dissenting voice in the Parisian scientific community, Pinel was convinced that all therapeutic efforts with the boy would be futile.[89]

Sensationalist psychology and idéologie. Once having declared for the efficacy of the moral treatment in those cases of insanity with "moral" causes, Pinel was obliged, as part of his scientizing project, to specify the mechanisms of both cause and cure. For this purpose he enlisted the aid of philosophy – hence the adjective *médico-philosophique* in the title of the *Traité*. His brand of philosophy, as *médecin éclairé* and member of the salon of Madame Helvétius, was of course sensationalist psychology: We have already noted that it was in profound ruminations on Locke and Condillac that he located his own superiority as a healer over the unreflective practices of the lay concierges. Pinel embraced as well the emended version of Condillac given by his contemporaries and friends the Idéologues; and what he referred to as *idéologie* – literally, the science of ideas – was the whole strand of thought from Locke and Condillac through the post-Revolutionary Idéologues.

[87] Ibid., pp. 106–7, 111–12.

[88] Cabanis, *Rapports, O.P.*, Vol. 1, p. 587.

[89] See Pinel's 1800 "Rapport fait à la Société des observateurs de l'homme sur l'enfant connu sous le nom de sauvage de l'Aveyron," published for the first time in *Revue anthropologique* 21 (1911): 441–54.

In the introduction to the first edition of the *Traité*, he made his allegiances and his intentions clear. He specified the works of the *idéologistes*[90] as guiding the "science of facts" that he envisioned his emerging study of insanity to be. They provided the stationary point (*terme fixe*) from which one could begin to make sense of the ever-fluctuating chaos of insanity and to capture the distinctive features of the different classifications of lunatics.[90] *Idéologie* was an "accessory science"[91] to Pinel's study of insanity: He used the same term as Vicq d'Azyr had used for those bodies of knowledge outside medicine's own observational base with which medicine entered into a beneficial relationship. Pinel's fellow Idéologues were also fond of claiming Pinel's work on insanity as part of *idéologie*. Cabanis described Pinel as one "to whom *idéologie* will owe almost as much as will medicine." In his basic textbook for students, the *Eléments d'idéologie*, Destutt de Tracy "could not recommend Pinel's *Traité* too highly. By explaining how madmen reason falsely, he teaches wise men how they think. . . . It is physiologist–philosophers like Monsieur Pinel who will advance *idéologie*." And it was as a devotee, almost an addict of *idéologie* that the young Stendhal – admirer and personal friend of Destutt de Tracy, avid reader of Cabanis's *Rapports*, a man who regarded Condillac's *Logic* as a suitable gift for his mistress – heard about Pinel's *Traité* and turned up in the medical school library in quest of it.[92]

Within the *idéologiste* canon, the concept of imagination, given its fullest explication by Condillac, is most critical to Pinel's construction of the cause and cure of insanity. "Of all the faculties of the understanding," Pinel observed, "the imagination appears to be the one most liable to profound lesions."[93] This observation, which had the full backing of Condillac's psychology, has already been obliquely suggested by the tactic Pinel employed in the moral treatment of destroying a delirious idea by "strongly shaking the imagination." We must now further explore Pinel's philosophical rationale for this therapeutic procedure.

For Condillac, imagination was one of the earlier operations of mind to be generated from the first, simple act of perception. It was closely allied to memory, since both were powers of reviving past perceptions independently of the external objects which had initially given rise to them. Noting that Locke had confounded the two, Condillac went on

[90] *Traité*, 1st ed., Introduction, pp. xxxv, liv. *Idéologiste* was the original, self-referential name for a practitioner of *idéologie*; "Idéologue" was the coinage of a detractor, Chateaubriand, but it is the name that has survived. See Joanna Kitchin, *Un journal "philosophique": La Décade, 1794–1807* (Paris: Minard, 1965), p. 118 n6.

[91] *Traité*, 1st ed., Introduction, p. lii.

[92] Cabanis, *Rapports, O.P.*, Vol. 1, p. 587; Destutt de Tracy, *Eléments d'Idéologie*, 3d ed. (Paris: Courcier, 1817; facsimile reprint, Paris: J. Vrin, 1970), Vol. 1, pp. 299–300 n1; Del Litto, *Vie intellectuelle de Stendhal*, pp. 287–88.

[93] *Traité*, 1st ed., p. 70 (also R & O, p. 231).

to specify their differences. Memory required the use of "arbitrary" or "conventional signs" (*signes d'institution*) – that is, language – attached to old perceptions in order that they be retrieved. Imagination could use arbitrary signs, but it could also make do without them: Hence beasts have no memory, but they do have imagination. Imagination, in its capacity for nonverbal functioning, was the more "bestial" of the two operations. Its greater vividness and sensuous immediacy were attributable to the fact that it often "revives the perceptions themselves," while memory "only recalls the signs."[94]

Imagination had a second power, which Condillac added to his definition: It not only revived perceptions but, in so doing, made "new combinations of them at our pleasure." It was thus a potential danger to the sober working of the understanding, an obvious point of entry for error. It had the liberty to play, scramble, and embellish, to "transport the qualities of one subject to another," to "gather in one what for nature sufficed to adorn several" – in short, to dispose of our ideas in a manner "contrary to truth." Some of the linkages of the imagination were made voluntarily, some automatically under the influence of an external impression. (Condillac's example of the latter was the sight of a sharp precipice evoking the idea of violent death.) These automatic linkages were especially firmly cemented, and we were often powerless to separate them; we could, in effect, easily cease to be "masters" of imagination and instead become its "dupes," falling victim to "prejudices" based on merely circumstantial connections between ideas which appeared to be immutable principles. Imagination made us vulnerable to another source of error as well: We sometimes could not distinguish the imaginary objects in our minds from real objects. Condillac considered the example of daydreaming – building "castles in Spain," imagining ourselves fictional heroes or heroines. These were usually safe and innocent pastimes, but under the impact of chagrin and sadness, as we withdrew from ordinary social contact, such daydreams could become more firmly rooted, the power of our judgment to relegate them to the world of imaginary inventions could weaken. "Little by little, we will take all our chimeras for realities." Hence the danger of novel reading, especially for young women, who have little education and hence little capacity to distinguish the fictive from the real.[95]

It is from this discussion of imagination that Condillac derives his fundamental definition of insanity (*folie*): "an imagination which, without our being able to notice it, associates ideas in an entirely disorderly fashion and sometimes influences our judgments or our conduct." On

[94] *Essai sur l'origine des connoissances humaines*, in *Oeuvres de Condillac* (Paris: Ch. Houel, Year VI/1798), Vol. 1, pp. 57, 64, 75, 78–81, 88–9. There is concrete evidence that Pinel read this particular text of Condillac. He cites the *Essai* in his *Nosographie philosophique, ou la méthode d'analyse appliquée à la médecine*, 6th ed. (Paris: Brosson, 1818), Vol. 3, p. 20.

[95] Condillac, *Essai*, in *Oeuvres*, Vol. 1, pp. 119–20 n1, 119–29.

the basis of this broad definition, Condillac jovially acknowledges that "it is probable that no one will be exempt"; we are all a little bit mad. But the difference between the fundamentally sane and the mad individual is that in the former the eccentricities of imagination are marginal, rarely intruding into the ordinary course of life, while in the latter, they are central and put the victim "visibly in contradiction with the rest of mankind."[96]

Pinel's use of theatrical devices to "shake the imagination strongly" as an integral part of the moral treatment is now more fully comprehensible and theoretically grounded. An insanity viewed as an imagination gone awry can be countered by a procedure that "shakes up" the imagination in order to dislodge the erroneous idea that has taken hold or to rupture the "vicious chain of ideas."[97] Theatrical display – the *appareil* – is used for this purpose rather than logical argument because it can be assumed that a disordered imagination is operating on a primitive, nonverbal level and can be effectively addressed only on that level. Condillac stated explicitly that to the extent that imagination works by means of linguistic signs, the individual exercises control over it, and rationality prevails.[98] In this regard Pinel raised indirectly, but did not pursue, the issue of whether a jolt to the imagination could subsequently be translated into verbal terms and understood rationally by the patient, or whether the theatrical fiction had to be perpetuated in order that its salutary effect be retained. In the case of the guilt-ridden tailor, he ascribed the eventual therapeutic failure partly to "the imprudence of someone's having mentioned to [the patient] that the definitive sentence pronounced on him during his 'trial' was a mere practical joke."[99] Was the tailor's reason still insufficiently "fortified" at that moment to make the transition from the imagistic to the verbal, or could such a transition never be made? Pinel, the expositor of the moral treatment, does not say.

In its deliberate use of spectacle to address the imagination and alter its contents, the moral treatment bore a strong family resemblance to a contemporaneous phenomenon, the *fête révolutionnaire*. The organizers of these civic celebrations during the Revolutionary decade operated within the same sensationalist-psychological framework as Pinel. They believed in the power of sensory impressions, and of visual images in particular, to "imprint" the "soft wax" which was their characteristic metaphor for the mind. One of their goals was to create, through sensory bombardment, an imaginative connection between the idea of the Republic and that of a superabundant richness, a connection that they believed would produce unwavering political commitment and an

[96] Ibid., pp. 130–31.
[97] Phrase from *Traité*, 1st ed., p. 58 (also R & O, p. 223).
[98] Condillac, *Essai*, in *Oeuvres*, Vol. 1, p. 86.
[99] *Traité*, 1st ed., p. 237.

invincible heroism.[100] The *fête révolutionnaire,* then, sought to establish politically salutary linkages in the imagination, the moral treatment to sever medically pathological ones.

A school for the passions. But *idéologie* did not embrace the full scope of the moral treatment, for as has been noted, *le moral* had two distinct components – ideas and passions. "The analysis of the functions of the human understanding," wrote Pinel, "has no doubt been greatly advanced by the collected works of the *idéologistes,* but there is another analysis as yet hardly sketched out and for which the cooperation of medicine is needed" – that of the "moral affections" or passions.[101] Medicine was needed with particular urgency here because the passions were thought to belong to the organic life more intimately than did ideas. They arose from internal rather than external sensations, were "sensed" by bodily organs other than the senses. The epigastric region was supposed to be a kind of "home" (*foyer*) or congregating place for them, a visceral center where all passional impressions eventually impinged and where, according to their specific natures, they selectively and palpably altered digestion, respiration, circulation, excretion. From the epigastrium they were transmitted "by a kind of irradiation" or "sympathetic influence" along neural pathways until they reached the brain and, if they were violent enough, disrupted its functioning.[102]

Condillac, it should be said in his defense, was not entirely unmindful of the passions. He saw constitutionally rooted passions as involved in the operation of attention, determining the particular objects that attracted an individual and thus stimulating his or her capacity to attend selectively. Passions were likewise importantly involved in the operation of imagination, exciting it into action, sometimes coloring its products so that they acquired greater verisimilitude and, in extreme cases, became indistinguishable from reality. Condillac went so far as to observe that, for a full consideration of the workings of the mind, an analysis of the operations of the understanding was insufficient; the passions had also to be examined and notice given to "how all these things" – the ideational and the passional – "are combined and confounded." Nonetheless, he felt obliged to exclude a sustained study of the passions from his own work on the grounds that it would be unmanageable and premature.[103]

[100] On these points see Mona Ozouf, *La fête révolutionnaire, 1789–1799* (Paris: Gallimard, 1976), pp. 241–45.

[101] Introduction to *Traité,* 1st ed., p. xxxv.

[102] This depiction of the passions is a composite of scattered comments by Pinel – including Introduction to *Traité,* 1st ed., pp. xxxvii–xxxix; "Mémoire sur la manie périodique," p. 100; and *Nosographie philosophique,* 6th ed., Vol. 3, p. 53 – and the more focused discussion in Esquirol's *Des passions,* pp. 17–20.

[103] Condillac, *Essai,* in *Oeuvres,* Vol. 1, pp. 50, 67, 149, 154.

Destutt de Tracy was also aware that an *idéologie* without the passions was seriously flawed. In 1796, he drew a distinction between two ongoing Idéologue enterprises. "Physiological ideology," the project undertaken by Cabanis and other physicians, "required vast knowledge," empirical data about the human organism still in the process of being collected. "Rational ideology," to which Tracy had devoted himself, was by contrast a deductive-philosophical project and "demanded less knowledge."[104] Regarding the two projects as complementary, Tracy tried to incorporate Cabanis's findings into his own work, stating in the *Eléments* that "internal sensations" as well as external ones "set our sensitivity to work" and identifying "passions properly so-called" as "true internal sensations." Other "affections" could derive either from internal sensations (the joy caused by good wine) or from ideas (the joy caused by good news). Yet, however well-intentioned, this consideration of the passions and allied feeling-states remained cursory. As Tracy confessed at the very end of the volume: "This is, I believe, what constitutes *idéologie.* I only regret not having linked it more closely to physiology; but that would have been to go beyond the limits both of my plan and of my own expertise."[105]

Pinel was, in other words, quite correct in his assessment that *idéologie,* at the hands either of its precursor Condillac or its literal founder Tracy, was inadequate to guide his own examination of the passions as causes of insanity or as curative agents to be deployed in the moral treatment. Where then could he seek guidance? Most immediately in the work of an English physician, Alexander Crichton, to whose *Inquiry into the Nature and Origins of Mental Derangement* (1798) he frequently acknowledged his debt.[106] The *Inquiry* contained a long discussion of the passions, apparently tailor-made to Pinel's needs, for it was written from the joint viewpoint of sensationalist psychology – Crichton was familiar not only with "our British psychologists such as Lock [sic], Hartley, Reid, Priestley, Stewart," but also with the work of Condillac – and medicine. Indeed Crichton believed his own originality to lie in the fact that while "moralists and metaphysicians have written copiously on the subject [of the passions]" and their implications for good and evil, he was the first to consider them with the impartial "eye of a natural historian" as simply a "part of our constitution."[107] In

[104] Cited in Staum, *Cabanis,* pp. 171–72.

[105] Tracy, *Eléments,* Vol. 1, pp. 33–37, 424. Staum points out that Tracy oversimplified Cabanis's position on the nature of the passions; see *Cabanis,* p. 239.

[106] See esp. the long section on Crichton (or Crighton, as Pinel usually misspells it) in *Traité,* 1st ed., pp. xxi–xxvii, xxxv–xl; and the reference to Chrigton [sic] in *Traité,* 2d ed., p. 12.

[107] Alexander Crichton, *Inquiry into the Nature and Origins of Mental Derangement* (London: Cadell, Jr., & Davies, 1798; facsimile reprint, New York: AMS Press, 1976), Vol. 1, pp. xxvii–xxxviii; Vol. 2, pp. 98–99. Pinel explicitly praised him for taking this approach; see *Traité,* 1st ed., Introduction, p. xxii.

separate chapters on joy, melancholy, fear, anger, and love, Crichton discussed the bodily and ideational sources of each, their effects on the various faculties of the mind and on the body, and the continuum along which their more intense forms were converted into different types of mental pathology.

Crichton offered Pinel the most systematic discussion of the passions to be found anywhere in the scientific literature. But Pinel was quick to perceive a particular inadequacy in it; and he thereby revealed the identity of his other, more essential mentor on the subject of the passions. Crichton held that passions were secondary phenomena, arising when the primary phenomena, the desires (e.g., hunger, thirst, sex)–which had clear-cut biological causes and loci and which ministered to the indispensable goals of preservation of the organism and its perpetuation through reproduction–met with obstacles in their drive for satisfaction. It was these thwarted desires, the passions, which had definite pathogenic potential, not the desires themselves. Pinel accepted this primary–secondary distinction, but he questioned the alleged naturalness of all desires and insisted that a more nuanced taxonomy be supplied. The English author, he maintained, "should have added that social life . . . extends almost without limits the needs relative to existence, that it ushers in our concern to gain the esteem of other men, honors, dignities, wealth, fame–and these are artificial desires (*désires factices*) which, always in a state of excitement and rarely satisfied, often lead to the destruction of reason."[108]

Pinel supplied no source for these ideas, but their Rousseauean inspiration is clear. What Pinel has in fact presented here is a kind of capsule summary of the *Discourse on the Origin of Inequality*, with its emphasis on the malign *sentiment factice* of amour propre introduced into the human condition by the founding of society, and its passing comments, so consistent with the fundamental psychophysiological postulate of the Idéologue physicians, interpreting disease as largely a by-product of social life, fostered by "immoderate transports of all the passions."[109] Doubtless Pinel did not think, in this emendation of Crichton, to cite Rousseau by name. Explicit citation was superfluous, because Pinel was mentally saturated with Rousseau, a situation typical of the entire generation of Idéologues.[110] Indeed, Pinel's involvement with Rousseau was probably even greater than average. According to a legend current by the early nineteenth century, he had made a

[108] Crichton, *Inquiry*, Vol. 2, pp. 110–14; and Pinel, *Traité*, 1st ed., Introduction, pp. xxv, xxvi n1.

[109] J.-J. Rousseau, *Oeuvres complètes* (Paris: Gallimard-Pléiade, 1959–), Vol. 3, p. 138 and Rousseau's note 15, p. 219.

[110] See Marc Regaldo, "*La Décade* et les philosophes du XVIIIe siècle," *Dix-huitième siècle* 2 (1970): 113–30; and Jean Roussel, *Jean-Jacques Rousseau en France après la Révolution, 1795–1830* (Paris: Armand Colin, 1972), Part 1, Ch. 2.

pilgrimage to the tomb of Rousseau with his friend Chaptal in 1778; though he was unable to sleep for the entire five days and nights of the foot journey, his spiritual exhilaration was such that he returned to Paris fully alert and without a trace of fatigue.[111] To the Idéologues, Rousseau was primarily the great expert on the passions, so it is hardly surprising that Pinel should turn to him automatically when faced with an intellectual problem concerning the passional life. Pinel was, furthermore, well aware that Rousseau had intimate, introspective knowledge of this subject matter: He cited the Rousseau of the "last two parts of the *Confessions* and the *Reveries of a Solitary Walker*" as a case study in melancholia, illustrating the frequent connection of that malady with a fearful mistrust of other people and the abiding conviction that everyone is an enemy.[112] Fittingly, then, through the vehicle of Pinel's implicit and diffuse Rousseauism, Rousseau, so full of psychological suffering and insight himself, became a distantly presiding deity over the birth of French psychiatry.

Pinel gave, as we have seen, a shorthand version of the strategy of coping with diseased passions in the moral treatment – not to destroy them, but to counterbalance them with more powerful passions. This formula seems to have derived from both Enlightenment and ancient sources. At about the middle of the eighteenth century, a noticeable shift occurred in Enlightenment concepts of mind. In a substantial series of texts – including Rousseau's second *Discourse* – passion, once depicted primarily as the subverter of reason, came to be seen in a positive light as reason's indispensable partner.[113] This reevaluation of the passions lies behind Pinel's refusal to control disordered passions by destroying them, for to do so would be to strike at the core of the organism's vitality. As Diderot put it in 1746, the deliberate "ruination of the passions" was "the height of ridiculousness," since, if it succeeded, it would produce "a true monster" who "desired nothing, loved nothing, felt nothing."[114] At the same time, Pinel's proposal to use the device of "counterbalancing" to harness the vital energy of the passions while curtailing their pernicious effects seems to be Aristotelian in inspiration, derived from the definition of virtue in Aristotle's *Ethics* as the mean between an excess and a deficiency of a passion.[115]

[111] E. Pariset, "Eloge de Ph. Pinel," *Histoire des membres de l'Académie royale de medecine* (Paris: J.-B. Baillière, 1845), Vol. 1, p. 256.

[112] Pinel, *Nosographie philosophique*, 6th ed., Vol. 3, p. 95.

[113] Ernst Cassirer, *The Philosophy of the Enlightenment*, trans. F. C. A. Koelln and J. P. Pettegrove (Princeton, N.J.: Princeton University Press, 1951), pp. 107–8.

[114] Diderot, *Pensées philosophiques*, in *Oeuvres complètes*, J. Assézat, ed. (Paris: Garnier, 1875–77), Vol. 1, p. 128.

[115] Kathleen M. Grange, "Pinel and Eighteenth-Century Psychiatry," *Bull. Hist. Med.* 35 (1961): 442–53, makes this suggestion (p. 443), an entirely plausible one in light of Pinel's tendency to incorporate classical precepts into his medical science. An alternate source for Pinel's strategy might be inferred from Albert O. Hirschman, *The*

But while Pinel articulates this model of counterbalancing quite explicitly, he rarely applies it in his actual case studies of the moral treatment.[116] The model widely if tacitly applied in Pinel's "exemplars" is that of Rousseauean schooling – both the total pedagogy of the *Emile* (the Idéologues' favorite text of Rousseau)[117] and the distinctive mode of adult "reeducation" described in the popular epistolary novel *La nouvelle Héloïse*.[118]

The ideal education described in detail in the *Emile* is centered on the passions and on monitoring their development. Underlying it is Rousseau's assumption that man in society is alienated from his true, natural self and hence, in the words of one recent commentator, "requires a healing education which returns him to himself."[119] Rousseau calls it a "negative education" because its main goal is to prevent the emergence of the artificial social passions – especially amour propre – and as a corollary to avoid the precocious stimulation of imagination, which as the second *Discourse* had pointed out is the faculty that sabotages man's moral progress, turning it into the source of his misery.[120] To achieve this goal, the child must be withdrawn from society at large and placed in a controlled environment which simulates the "natural" situation. There he receives the constant ministrations of a full-time tutor, who in order to be effective must have complete and absolute authority over his charge. The tutor, paradoxically, works through elaborate and even deceitful artifice to restore "naturalness." He carefully structures the environment, controls events, and prearranges experiences so that, while never overtly giving orders, he is omnipresent and compels his pupil to learn. The noble estate owner Wolmar in the

Passions and the Interests: Political Arguments for Capitalism Before Its Triumph (Princeton, N.J.: Princeton University Press, 1977), pp. 14–31, which contends that the search for a secular method of controlling the passions was common to most European thinkers of the seventeenth and eighteenth centuries. Hirschman discerns three proposed methods: to repress the passions, to harness them for useful ends, and to "countervail" a harmful passion with a less harmful one. The last is virtually identical to Pinel's "counterbalancing."

116 Nor was Pinel absolutely consistent in his support of a nondestructive "balancing." He urged, for example, that cured maniacs read Plato and the Stoics as a "moral remedy" to guard against relapse by helping them "to vanquish [the] passions." *Traité*, 1st ed., p. 36.

117 Roussel, *Rousseau après la Revolution*, p. 41. The Idéologues favored the *Emile* for methodological reasons, regarding it as a work of "analysis."

118 The parallel between Emile's tutor and Wolmar in the *Nouvelle Héloïse* is made very forcefully by Judith N. Shklar, *Men and Citizens: A Study of Rousseau's Social Theory* (Cambridge University Press, 1969), Ch. 4, esp. pp. 146–50. As far as I know, Pinel never cited the *Emile* by name, but he clearly alluded to that pedagogical text in *Nosographie philosophique*, 6th ed., Vol. 3, p. 160.

119 Allan Bloom, Introduction to Jean-Jacques Rousseau, *Emile, or On Education*, trans. A Bloom (New York: Basic Books, 1979), p. 3.

120 Ibid., pp. 7–9; on the "superfluity" of imagination, see also *Emile*, in *Oeuvres complètes*, Vol. 4, pp. 304–5.

Nouvelle Héloïse uses artifice in an analogous fashion to cure Saint-Preux of his disordered and disabling passions – constant self-torment by regret and nostalgia. Wolmar and Emile's tutor are both images and embodiments of godlike paternal authority – figures typical in Rousseau's work – who serve as levers capable of rupturing and reversing the vicious trend of human history and raising man out of the corrupted social condition.[121]

The parallels with the moral treatment will be immediately apparent. Both Rousseauean schooling and Pinellian healing devolve upon an all-powerful, larger-than-life authority figure. (The lunatic would be cured, said Pinel, by putting him in a relationship of "strict dependence on a man who, by his physical and moral qualities, is able to exercise an irresistible authority over him.")[122] This authority tends to be vividly embodied. Much like the concierge Francis Willis, Wolmar has a penetrating eye, sometimes called his *oeil éclairé*, with extraordinary powers of looking into the hearts of others.[123] Both Rousseauean schooling and Pinellian healing require a controlled environment withdrawn from the larger society. ("The most powerful remedy . . . can only be found in a well-ordered hospice.")[124] And both call for astute management of the passions (Pinel proves, wrote Destutt de Tracy, that "the art of curing madmen is the same as that of governing the passions of . . . ordinary men"),[125] in both cases achieved through the use of contrived situations, artifice, and pious fraud.

Even the Rousseauean motif of alienation from and restoration to one's true nature is completely applicable to Pinel's therapy. For not only was insanity by its very name "mental alienation" (the formal term *aliénation mentale* had, in medical discourse at least, generally replaced the vulgar *folie*),[126] but Pinel's rhetoric typically depicted cure as an overcoming of alienation, a return to one's true self, a *retour sur lui-même*, and an act of being *ramené à lui-même*.[127] Such terminology is also used in the *Nouvelle Héloïse* to describe the effects of Wolmar's "therapy": "He return[ed] me to myself in spite of myself," his wife

[121] Shklar, *Men and Citizens*, Ch. 4, pp. 127–50.
[122] *Traité*, 1st ed., p. 58 (also R & O, p. 223).
[123] Shklar, *Men and Citizens*, pp. 135, 143.
[124] *Traité*, 1st ed., p. 58 (also R & O, p. 223).
[125] *Eléments*, pp. 229–300 n1.
[126] The terms *aliénation mentale*, *aliénation d'entendement*, *aliénation d'esprit* or simply *aliénation* entered French usage as synonyms for *folie* in the seventeenth century in both medical and literary writing; see Paul Robert, *Dictionnaire alphabétique et analogique de la langue française*. Pinel was strongly in favor of the adoption of *aliénation* and the abandonment of *folie* in medical discourse, seeing this linguistic purification as a necessary part of the effort to make medicine more like "the other sciences"; see *Traité*, 2d ed., pp. 128–29, and the discussion of language and psychiatric professionalization in Chapter 4 of this volume.
[127] *Traité*, 1st ed., pp. 59, 65 (also R & O, pp. 224, 228).

gratefully testifies, recalling the earlier chaotic state of her passions, which had made her "experience myself as an 'other' outside of myself."[128] In the second edition of the *Traité,* Pinel even noted this parallel between the moral treatment and pedagogy: "What an analogy there is between the art of directing lunatics and that of raising young people! Both require great firmness, but not harsh and forbidding manners; rational and affectionate condescension, but not a soft complaisance that bends to all whims." It surely was not an accidental detail in Pinel's account of the case of the furious soldier that the patient, at the moment that he bows to medical authority, begins to *tutoyer* the chief physician of the hospice; his childish language is emblematic of the status of a child, to which he has, in the process of being cured, reverted.[129]

The Rousseauism that shaped the moral treatment spilled over into other areas of early psychiatric speculation, contributing most notably to the construction by Pinel, Cabanis, and Esquirol of a grand theory about the relationship of insanity to the stage of development of human civilization. The theory hinged upon the *passion factice,* which was a time-bound cultural product, summoned into existence by the birth of society and sensitive to historical change, growing in strength as society became more advanced, complicated, and "unnatural." The inherent insatiability of the *passion factice* made it prone to take on excessive and unruly forms, and these in turn readily conduced to insanity. Pinel's suggestive pronouncements on the pathogenic tendencies of the *passion factice* gave way, at the hands of his student Esquirol, to a series of environmental correlations. Among people who were *policés,* insanity of all types proliferated luxuriantly because of the preponderance of "vehement" and "impetuous" *passions factices;* hence cities more than countryside, capitals more than "cities of the second rank" were nurseries of insanity.[130]

Writing during the early years of the Revolution, Cabanis gave this theory a political–and messianic–dimension. He put his faith in the ability of a new republican political constitution "founded on human nature" to diminish the incidence of insanity and all other disorders of the mind. In the regenerated French polity, he predicted confidently, "society will no longer degrade man; . . . it will no longer stifle in him the passions of nature in order to substitute *passions factices* . . . suitable only for corrupting reason and habit." Citing Rousseau directly, Pinel expressed similar epidemiological views in a medico-political letter to

[128] "[Il] me rend à moi malgré moi-même," *La Nouvelle Héloïse,* Part 3, Letter 18, in *Oeuvres complètes,* Vol. 2, p. 356. "Je me sentis tout autre au dedans de moi," ibid., p. 351. The first passage is cited in Shklar, *Men and Citizens,* p. 143.

[129] *Traité,* 2d ed., p. 20; *Traité,* 1st ed., p. 59 (also R & O, p. 224).

[130] Esquirol, *Des passions,* pp. 14–15.

the editor of 1790. The nervous and chronic diseases rampant in France in the twenty or thirty years before the Revolution were, he said, characteristic of a "social order ready to expire," one in which people were mired in "softness and luxury," social bonds had slackened, and "self-interest had frozen all hearts." But the Revolutionary rejuvenation of society – the progress of liberty, "the development of what is called public spirit" – had literally "salutary effects" that were unmistakable to the physician. Within each Frenchman, the expansion of "the soul" produced by political events had redounded to the benefit of the "animal economy," lending it a "surplus of vigor and energy." "People can be heard to say, 'I feel better since the Revolution.' "[131]

When the period of revolutionary zeal was past, Pinel and Esquirol turned their Rousseauism to more conservative ends. Now they tended to view political upheaval as *increasing* the incidence of insanity precisely because it massively stimulated all the passions, and "gave wings" to the artificial ones related to interpersonal rivalries and the loss or gain of wealth and position. A numerical increase in the lunatic population had, said Esquirol, been "observed in France since the beginning of our revolutionary torment." And Pinel noted in 1805 that since the data in his *Traité* had been gathered during "the most stormy epochs of the Revolution," when passions were most inflamed, his assertions about the causes and treatment of insanity remained to be verified in normal times – one of the purposes of his then-current research at the Salpêtrière.[132]

Statistics. The final way in which Pinel sought to elevate the moral treatment to fully scientific status was through the application of statistical methods. The efficacy of the treatment, explained by reference to philosophy, was now to be measured and confirmed quantitatively. Clinical observation was for him (as for Vicq d'Azyr) the basis of medical scientific knowledge; but while he thought such observation entirely adequate for the relatively advanced task of classifying diseases, he felt it left much to be desired in the area of therapeutics. The foundations of this latter part of medicine were "shaky" and its contents only "vague precepts"; "everyone vaunts his own results and more or less cites facts in his own favor." Therapeutic experiments could not continue to be performed in an ad hoc, impressionistic manner, but in order "to be authentic and conclusive . . . must be made on a large number of patients." Cases in which the treatment had failed could not simply be omitted – to preserve an illusion of the physician's extraordinary powers or to avoid wounding his pride – but had to be frankly included in the

[131] Cabanis, "Vues sur les secours publics," *O.P.*, Vol. 1, p. 59; Pinel, "Aux auteurs du journal," *Journal de Paris*, January 18, 1790, p. 71.

[132] Esquirol, *Des passions*, p. 15; Pinel, "Recherches sur le traitement général des femmes aliénées," *Moniteur universel*, June 30, 1805, p. 1158.

report and preferably even analyzed at length, for by ascertaining what had gone awry, progress in the application of the treatment could often be made.[133] There had to be a recognized court of last resort in therapeutics lest "contrary opinions" be pitted against each other in an "eternal struggle."[134] In short, the compilation of statistics had to supplement clinical observation; the therapeutic enterprise of medicine "can take on the character of true science only by the application of the calculus of probabilities."[135]

Pinel embarked upon his statistical project around 1800, as soon as he received at the Salpêtrière the patient population of curable insane transferred from the Hôtel-Dieu. This much is apparent because in 1802 at a public session of the Ecole de médecine de Paris, the presiding officer, surveying the recent achievements of members of the faculty, revealed Pinel's interim statistics on the success of his treatment during the Year X.[136] But only in 1807, in an address before the Institut, did Pinel finally report fully on the four-year statistical "experiment" he had conducted. Its purpose was to decide which of the two competing methods then used to treat insanity was preferable: the traditional one, which "consists in hastening the malady on its course through repeated bleedings, strong showers, cold baths or even surprise baths"; or "the other . . . adopted at the Salpêtrière," which was essentially a combination of the moral treatment with what Pinel elsewhere called the "method of expectation" – allowing the malady simply to run its course on the assumption that a spontaneous recovery would ensue, but at the same time watching the patient assiduously so that the healing powers of nature could be supported and furthered by medical intervention of a "moral" or physical sort.[137] Pinel emphasized that he had spared no pains in rigorously carrying out his experiment. He had systematically collected data about each of his Salpêtrière subjects. He had delimited the timespan of the experiment and specified as clearly as possible the "elements" of the "Salpêtrière method" (the aspect of expectation remained constant, but the active remedies varied according to the specific case of insanity and could not be prescribed a priori). And he had made careful tabulations and scrupulously identi-

[133] Pinel, *Résultats d'observations et construction des tables pour servir à déterminer le degré de probabilité de la guérison des aliénés* (Paris: Baudouin, 1808), pp. 1, 1–2 n1. Pinel was adamant much earlier in his career about honest reporting of the failed case; see *Traité*, 1st ed., p. 47 and pp. 53–54, where he criticizes Willis on this account (also R & O, pp. 216, 220–21).

[134] Pinel, "Recherches sur le traitement des femmes aliénées," p. 1160.

[135] *Probabilité de la guérison*, p. 2 n1.

[136] Hallé, "Discours," p. 26. Another brief interim statistical report was offered by Pinel himself in 1805 in "Recherches sur le traitement des femmes aliénées," p. 1160.

[137] *Probabilité de la guérison*, pp. 5, 17–19. On the "méthode d'expectation," see *Traité*, 1st ed., p. 84 (also R & O, p. 241); and Pinel, "Expectation," *Dict. des sci. med.*, Vol. 14 esp. p. 252.

fied the "doubtful" cases (those whose prior history was unknown and those who had not been unequivocally cured).

The result of Pinel's study (in which there was no control group) was a 93 percent rate of cure for maniacs and melancholics treated by the "Salpêtrière method" whose illness was "of recent date and not treated elsewhere." Idiots, demented patients, and madwomen of long standing had been excluded from the sample as generally not susceptible to cure; so had those whose illness had been subject to a prior treatment, since they introduced unknown and imponderable factors into the experiment. This excluded population was of significant size: The 93 percent cure rate referred to less than half of the patients admitted to the Salpêtrière during the four years under consideration; and Pinel openly acknowledged that his cure rate for the Salpêtrière population as a whole was only 47 percent. But this was not, to his mind, a damaging concession because he regarded determining the population on which a therapeutic method is efficacious as part of the scientific task of the physician. He was confident that his excellent results with the delimited group could be replicated in any hospital that followed the same principles of treatment.[138]

Why was Pinel so wedded to statistics in his effort to render the moral treatment, as an instance of medical therapeutics, scientific? His own rhetoric provides us with part of the answer. By referring to the method he employed as the "calculus of probabilities," he placed himself, his statistical commitment, and his conception of science squarely in the camp of the mathematician–philosopher and progenitor of the Idéologues, Condorcet. As a recent study of Condorcet has shown, the thinkers of the late Enlightenment had come to regard probability theory as the essential building block of a science of man. An eighteenth-century philosophical commonplace held that, by contrast to the absolute certainty of mathematical knowledge, "knowledge" of the human world was defective, uncertain, merely probable. Apart from and initially unrelated to this epistemological lament, eighteenth-century mathematicians from Bernoulli to Laplace were developing, by reference to dice throwing and games of heads-and-tails, a sophisticated theory of probability. Condorcet's position was based upon the merger of the philosophical and the mathematical notions of probability. He was confident that skepticism could be overcome and that the "moral sciences" could achieve the same degree of certainty as their physical counterparts (though never the perfection of mathematical science) as soon as the calculus of probabilities became their language. The calculus would then measure on a sliding scale the accuracy of propositions about the human world; it would provide a

[138] *Probabilité de la guérison*, pp. 3, 23–24, 36–37.

sound basis for opinion and a "sure means of arriving at very great probability in some cases."[139]

Condorcet put these ideas into practice in his *Essai sur l'application de l'analyse à la probabilité des décisions rendues à la pluralité des voix* of 1785. His political science took the form of a mathematical rationale for representative government, a demonstration of the probability that, in an assembly, the vote of the majority would arrive at the truth and the laws so enacted would be "correct."[140] Analogously, Pinel attempted to construct a mathematical and therefore epistemologically sound science of medical therapeutics. His 1807 address to the Institute cited Bernoulli as one of his mentors in the application of the calculus of probabilities. In fact, Pinel's quantification, despite the impressive label he affixed to it, was quite simple – infinitely simpler than Bernoulli's or Condorcet's – and consisted merely in the arithmetical operation of finding the ratio between the number of lunatics cured and the total number of lunatics included in the experiment. But this in no way deterred him from presenting (or perceiving) himself as a disciple of Bernoulli, for as he pointed out, his approach relied upon "one of the elementary principles" of probability theory – "that the probability of an event is measured by a fraction whose numerator is the number of favorable cases and whose denominator is the number of all possible cases."[141]

If the direct intellectual influence prompting Pinel's turn toward statistics was Condorcet's conception of a human science, a more diffuse political influence moved him in the same direction. The period from the Directory to the Consulate, from the Year IV to 1804, was a "golden age" of bureaucratically inspired statistics-gathering, with circulars emanating from the Ministry of the Interior calling on the departments to inventory almost every aspect of local social and economic life, including the medical condition of the population. Since state administrative personnel were lacking for a project of this magnitude, the prefects were to enlist the collaboration of local savant societies and physicians, teachers, and engineers. This initiative, part of the effort to consolidate political control – as had been the statistical proclivity of the enlightened absolutists and, indeed, all French statistical inquiries since the time of Charlemagne's capitularies – was most intense during the periods (the summer of 1797 and June 1798–June 1799) when the philo-

[139] Keith Michael Baker, *Condorcet: From Natural Philosophy to Social Mathematics* (Chicago: University of Chicago Press, 1975), Ch. 3 passim, and esp. p. 183 for quote from Condorcet.

[140] Ibid., pp. 228–31.

[141] Pinel, *Probabilité de la guérison*, p. 4; Pinel explicitly mentions Bernoulli's *Ars conjectandi*. A discussion of Pinel as one of the founders of French medical statistics can be found in Terence D. Murphy, "Medical Knowledge and Statistical Methods in Early Nineteenth-Century France," *Medical History* 25 (1981): 301–19.

sophically inclined François de Neufchâteau held the post of minister of the interior.[142] He was the same minister to whom Pinel had sent Pussin's memoir on the moral treatment, together with his own request that Pussin be transferred to the Salpêtrière; and since these documents were found among François de Neufchâteau's personal papers,[143] it seems likely that there was some degree of personal contact between the minister and Pinel. In 1801, when Pinel had just embarked upon the statistical "experiment" at the Salpêtrière, his close friend Chaptal became minister of the interior and made a great contribution to statistics by sponsoring the publication and diffusion of the statistical labors commissioned by François de Neufchâteau.[144] To collect and deploy statistics, then, was manifestly to be in tune with and potentially useful to the government; and Pinel, who owed to the government the brilliant upswing in his career since the Revolution, could not have been oblivious to the political connotations of statistical method – or to the promise it held of continued political rewards.

A therapy for the Revolution

The odyssey of the moral treatment – its origins in "charlatanistic" practice; the salvaging operation by which Pinel converted it into a respectable tenet of official medicine, assimilating it to the models of sensationalist psychology and Rousseauean pedagogy and confirming it statistically – does not in itself explain its success as an originative paradigm. Kuhn offers some guidance here by providing two "internal" criteria for a paradigm. The moral treatment, it is not difficult to see, meets both of them. It was in the first place "sufficiently unprecedented to attract an enduring group of adherents"[145] – unprecedented, that is, in the context of official medicine, where the remedies traditionally sanctioned were of a physical nature. This fundamental novelty of the moral treatment was often remarked upon. Reviewing the first edition of the *Traité*, Moreau de la Sarthe described as "limited" the mentality of the traditional physician "for whom all the resources of the art are contained in the pharmacist's boutique. . . . The true physician will study the work of Citizen Pinel and will see the sphere of activity of the medical art enlarged." The young Esquirol described the moral treatment similarly as "pushing back (*reculer*) the limits of a science." Even in the mid-nineteenth century the moral treatment, as

[142] See Jean-Claude Perrot, *L'age d'or de la statistique régionale française (an IV–1804)* (Paris: Société des Etudes Robespierristes, 1977), pp. 5, 24–28, 61; and Pierre Marot, *Recherches sur la vie de François de Neufchâteau* (Nancy: Berger-Levrault, 1966), pp. 279–329.

[143] Weiner, "Apprenticeship of Pinel," p. 125.

[144] Perrot, *Age d'or*, p. 33.

[145] Kuhn, *Structure*, p. 10.

opposed to the usual practice of physicians who "exhaust the entire pharmaceutical arsenal," was being depicted by some as "this *new* way open to science."[146]

The second of Kuhn's criteria was also easily met: The moral treatment not only created a new scientific community bound by fundamental consensus but was "sufficiently open-ended to leave all sorts of problems for the . . . group of practitioners to resolve" and hence could usher in a period of "normal science."[147] Pinel had adumbrated the new approach, provided case studies and some implicit theoretical scaffolding, but many details of the new therapy remained to be worked out. He himself began this process by exempting dementia and idiocy – two of the four genres of insanity in his classificatory scheme – from the putative scope of efficacy of the moral treatment; he also exempted certain subspecies of mania.[148] But his scattered yet crucial suggestions that the tactics of the moral treatment had in every instance to be tailored a posteriori to the causes and precise ideational content of the delirium[149] were never systematically developed. Pinel left no shortage of "problems . . . to resolve"; his disciples report that he even underscored the "open-ended," capacious nature of his work. "Although," one wrote in 1818, "the *Traité* of Monsieur Pinel has been translated into every language [and] is the manual of every physician who treats lunatics, the celebrated professor, with that candor that belongs only to genius, never ceases to invite his students to improve upon what he has begun."[150]

But Kuhn's internal criteria do not exhaust the subject of why the moral treatment was so roundly hailed. They ignore the "fit" of this prospective paradigm with the external and in particular the political environment. As we have seen, Pinel's favor with the Revolutionary government and his resultant prominence in the newly created medical establishment enabled the moral treatment to pass readily into orthodoxy and to be institutionalized as the approved therapy for the insane at Bicêtre and the Salpêtrière. But the connection between the Revolution and the moral treatment went deeper than these circumstances, penetrating into the very nature of the therapy itself.

The politics of douceur. At the very opening of the Revolution, the lunatic acquired a peculiar importance – both real and symbolic – because

[146] Moreau de la Sarthe, "Art de guérir," p. 464; Esquirol, *Des passions*, p. 13; E. L., "De la musique dans le traitement de la folie," *Gazette médicale de Paris*, 2d ser., 10 (February 12, 1842), my italics.

[147] Kuhn, *Structure*, p. 10.

[148] *Traité*, 1st ed., pp. 81–83 (also R & O, pp. 239–40).

[149] See, e.g., "Recherches sur le traitement des femmes aliénées," p. 1160; *Probabilité de la guérison*, p. 18.

[150] Unsigned "Avertissement" to "Série de fragmens sur l'aliénation mentale," *Journal générale de médecine* 62 (1818): 145.

his status happened to be enmeshed in a salient political issue. The royal lettre de cachet was to the revolutionaries a singularly blatant example of the exercise of arbitrary power that marked the Old Regime. The special prisons for victims of the lettres de cachet were thus suppressed at once, and the National Assembly appointed a committee to draft a law on the disposition of the illegally detained prisoners. The committee's report, presented in February 1790, divided the prisoners into four groups: those who had never been accused of a crime; those condemned without appeal; those against whom an arrest warrant had been issued but who had never been brought to trial; and finally, those locked up for madness. The last group was further subdivided into those who were truly insane and those about whom the allegation of madness was a deliberate subterfuge. (It was standard Old Regime practice for one party who wanted another removed from the scene for whatever nefarious reason to obtain a lettre de cachet from the Crown authorizing incarceration on grounds of lunacy; by this device, parents punished disobedient children and greedy relatives tampered with the laws of inheritance.) Because of this admixture of real and alleged *fous*, the committee recommended that each member of the fourth group be visited several times by a medical doctor in order that the state of his reason might be accurately determined – a significant provision in itself, indicating as it does the Revolutionary scientism that fostered the medicalization of insanity. Most striking, however, were the remarks of the committee's *rapporteur* concerning the way the bona fide insane were to be handled:

> You will still, gentlemen, have to take actions to ameliorate the condition of the unfortunates who, needing daily surveillance, cannot enjoy liberty. Until the present, they have almost always been treated in the different *maisons de force* of the realm with an inhumanity which, far from curing their illness (*mal*), was suitable only to aggravate it. Persuaded that it is by *douceur* and not by the ferocity of a barbarous regimen (*régime*) that it is possible to cure these unfortunates, you will probably resolve to assign – either from the already existing funds of the *maisons de force* or from the [nationalized] ecclesiastical properties – a portion of revenues sufficient to assure to the insane the help that their condition requires of the public charity (*bienfaisance*).

He went on to say that this *douceur*, while necessary in all eras, was a "still more sacred obligation for us." For in opening the special prisons, it had become apparent that some of the prisoners whose reason had been completely sound when they were locked away, had become insane "by their long captivity and by the torments they suffered when the laws were mute and the king's ministers all-powerful."[151]

This passage, simple on the surface, is upon examination extraordinarily rich in its conflation of levels of meaning: The medical and the

[151] *Arch. parl.*, 1st ser., Vol. 9, p. 661.

political become almost indistinguishable. *Douceur* is held up both as a medical treatment and as a political desideratum. It is contrasted to the "ferocity of a barbarous regime" – where *régime* is equally a part of the medical and political vocabularies, and the "barbarous" regimen/regime rejected is simultaneously the micro-regimen of chains and other corporal punishments applied to the mad inmates of the *hôpital-général*, and the old political macro-regime with its willful disrespect for law and its "ministerial despotism."[152] *Douceur* in the medical treatment of the insane, as in politics, is thus the inevitable choice of the Revolution, and it becomes appropriate to espouse this form of medical treatment in a national legislative forum by dint of the analogies, almost equivalencies, posited: The care of the insane has become a thoroughly political issue. Having seen that *douceur* is a recurring word in Pinel's description of the moral treatment, we can now appreciate the political resonance which the word had for him and his readers.

A second and related aspect of the conjoint medical-political nature of insanity is touched upon here: insanity as a civil status entailing the deprivation of liberty ("the unfortunates who, needing daily surveillance, cannot enjoy liberty"). Such a status raises new and serious problems in a revolutionary setting where the foremost commitment is to liberty as the right of the individual citizen. The lunatic must be protected insofar as possible from the morally degrading consequences of unfreedom; the double "captivity" of an incarcerative institution and a despotic polity can, says the report, even cause insanity. Where deprivation of liberty is unavoidable, the committee offers *douceur* as compensation – as recognition of the right to liberty which, though abrogated, still resides potentially in the individual who has "lost the use of reason." The argument here about the political necessity for *douceur* is replicated (though relativized) in Pinel's 1798 reasoning about the kind of political consciousness that necessitates the moral treatment:

> It would perhaps be to fall into intellectual vagueness to treat in a manner that is general and uniform for all peoples the question of the institution of corporal punishment for lunatics. For how can we be sure that Negroes who live in servitude on Jamaica or Russian slaves, shaped by an oppressive system for the whole of their lives, ought not, in the case of insanity, be subjected to the same harsh and despotic yoke. But (leaving aside some favorable effects that can be expected from the use of fear in the cure of mania) does not the lively sensibility of the Frenchman and his violent reaction – as long as he conserves a glimmer of his reason – against every shocking abuse of power determine on his behalf the forms of repression that are the most gentle (*douce*) and the most in conformity with his character?[153]

[152] Ibid. for this phrase.
[153] R & O, p. 227 (also *Traité*, 1st ed., pp. 63–64).

And of all the forms of *douceur* possible in treating the insane, this line of analysis might continue, the Pinellian moral treatment is, in terms of the ideology of the liberal Revolution, the most politically "correct." A diffuse, nonspecific kindness would, to be sure, go some distance toward satisfying the criteria of political liberalism by recognizing the essential humanness of the lunatic and hence his capacity for freedom. This point was made in a bureaucratic admonition of the Year VI, under the politically liberal regime of the Directory, that *douceur* be employed in institutions for the insane: "How culpable, then, are those who, in familiarizing themselves with this malady, cease to see their fellow-creatures (*semblables*) in its poignant victims and treat them with harshness!"[154] But the specific strategies of the Pinellian moral treatment go beyond this; by striving to make contact with the vestiges of reason remaining in the lunatic, they ratify most forcefully his status as *semblable* and his consequent capacity for freedom. The developmental theories of Condillac and Rousseau that underlie Pinel's rendition of the moral treatment allow a bridging of the gap between the sane and the insane. They lay bare how reason went awry – by the assaults of an overstimulated imagination and the insatiable demands of artificial passions – and thus make possible the charting of a hypothetical path back to reason. The lunatic is not consigned to total "otherness" but is located on a continuum with the sane person, sharing the same mental structures. The moral treatment thus postulates the same attitude as Condillac's pedagogy, that of the feasibility of building, with the tools of sensationalist psychology, a bridge to a qualitatively different mind. "To execute my plan," wrote Condillac in his prefatory remarks to the course of study he outlined for the prince of Parma, "I must approximate (*s'approcher de*) my pupil, put myself entirely in his place; I must be a child rather than a tutor."[155] So too could the Pinellian practitioner believe himself able to reconstruct rationally the mental world of the lunatic, to "put [himself] in [the lunatic's] place," and, through this momentary act of identification, to lead him back to sanity.

[154] AN: AJ^2 60, "Circulaire du Ministre de l'Intérieur notifiant la réouverture de l'Hospice de Charenton," I nivôse Year VI. The "preliberal" conception of the lunatic as a being who had forfeited his humanness and reverted to animality was no liberal straw man. It survived residually even in the municipal police ordinances enacted by the National Assembly (the Laws of 16–24 août 1790 and 22 juillet 1791), and later in the Napoleonic civil code (Articles 475, 479), in provisions concerning penalties for those who had negligently "allowed madmen [*fous* or *furieux*] or wicked and ferocious animals in their keep to wander," either frightening or actually harming passersby or inflicting property damage. The insane individual and the wild beast were thus placed in a single category.

[155] Condillac, "Discours préliminaire" to *Cours d'études pour l'instruction du prince de Parme*, in *Oeuvres de Condillac*, Vol. 5, p. lxi.

The clamor for the moral treatment: non-Pinellian initiatives during the Revolution. Nothing makes clearer the incorporation of the moral treatment into revolutionary ideology and practice than the numerous attempts during the course of the Revolution to inaugurate the moral treatment – attempts that were wholly independent of the man who would eventually be credited with that accomplishment, Pinel. The first of these, which Pinel later cited in the introduction to the first edition of the *Traité*, was a 1791 pamphlet by an obscure civil servant in the department of the Var. Its message hinged upon the author's visit two years before to Père Poution, a monk and *guérisseur des fous* employing moral methods in the village of Manosque in the neighboring department of the Basses-Alpes. Mourre, the civil servant, described Père Poution in recognizably Rousseauean terms as a larger-than-life tutor – "a beneficent being who joins knowledge with *sensibilité*, [who] becomes the repository of [the lunatics'] pain and of their thoughts . . . , who goes back with them to the source of their ills . . . [and] breaks the spell of the illusions that surround them." Meeting some of the lunatics whom Père Poution has cured and moved by the depth of the gratitude they show for the monk, Mourre makes his resolve. "I said to myself: here a single man spreads so much benefit! and this man is ignored while the author of an epigram is known throughout France!" Addressing "my compatriots," he went on to present a scheme whereby *secours moraux* could be integrated into the treatment of lunatics throughout France without the expense of building a special hospital in each department. ("I aspire to reconcile . . . the calculations of economy with the duties of humanity.") The existing foundations – the hôtels-Dieu and the *hôpitaux-généraux* – were to remain, but their directors would work individually, in the fashion of Père Poution, with the lunatics dispersed among their inmate populations. Mourre explicitly assigned the moral treatment to lay personnel and gave to the accredited physician the function of attentively observing the course of the insanity and prescribing drugs. This projected new treatment program for the insane was an end to "barbarism" consonant with the Revolution, or as Mourre referred to it elliptically, the "history of the French nation . . . for almost the past three years."[156]

A similar proposal, also written in 1791, can be found in the archives of the revolutionary commune of Paris. It emanated from a certain Escourbiac, a surgeon and the proprietor of a Paris *maison de santé* – or as he called it, using the virtually synonymous term, a *pension bourgeoise* – which catered to patients of all types. Escourbiac's was one of the better known private facilities for lunatics in the capital, quite on a par in this regard with the *maison* Belhomme, which had once em-

[156] Mourre, *Observations sur les insensés* (Toulon: Impr. de Surre, 1791), pp. 5–6, 14, 19, 21–22.

ployed Pinel. In his manuscript memoir to the mayor of Paris, Escourbiac noted that he had always been especially interested in the treatment of *aliénés d'esprit* and had in his sanatorium made a study of the various modes of therapy that could be applied to them. He had achieved his most impressive results largely through "moral means rather than physical means." In light of the expertise he had acquired, which contrasted so sharply with prevailing techniques, he urged the formation of a new "therapeutic national and patriotic" establishment for the treatment of the insane where his principles would be followed. He would generously lend his services to such an establishment – by implication, he would also direct it – asking no salary but only government subsidies for food and equipment. At the end of his proposal. Escourbiac gave Thouret as a reference; thus it seems that he traveled in the same circles as Pinel. The surgeon, however, seems to have traveled very much on the periphery, for he was unable to secure the backing of those in power. While the reply of the mayor's office could "only applaud" the humanity of the suggestions Escourbiac had submitted, it indicated that the municipality would have to wait until the national legislature made good its promises to organize a broad system of public assistance before undertaking any such projects on its own.[157]

Yet a third plan for the incorporation of the moral treatment into a system of government-sponsored institutions was the subject of a memoir by a Monsieur Broutet, the director of a hospice for lunatics in Avignon. Unlike Escourbiac's, which ended up in the dim recesses of a bureaucratic dossier, Broutet's memoir acquired a certain celebrity when it received a medal from the Lycée des Arts in the Year V (1797) and was subsequently published and discussed in the *Décade philosophique*.[158] The Lycée des Arts, founded in Paris in 1793 by an engineer, was an expression of the antielitist, antiacademic ideology that preponderated during the Jacobin phase of the Revolution. Its primary aim was to raise the dignity of the artisan by freeing him from the clutches of the Academy of Sciences – giving him his own institutional center for the communication of his inventions and a series of prizes that would spur him on to achievement.[159] That the moral

[157] See AN: F[15] 242, no. 578. The memoir is undated, but textual evidence points to 1791 as its year of composition. On the prominence of the *maison* Escourbiac, see Tuetey, *Assistance publique*, Vol. 1, p. xvii and p. xvii n1.

[158] See *Mémoire qui a obtenu une médaille au Lycée des arts le 30 Messidor an 5 . . . contenant un exposé précis et succinct de la situation de l'Hospice des Insensés de la ville d'Avignon et du mode de traitment qui a operé des cures . . .* (Paris: Impr. du Lycée des Arts, n.d.), BN: Te[66] 91. This four-page booklet contains Broutet's memoir and the appreciation of it by the president of the Lycée des Arts. The *Décade philosophique* article, largely an abridgment of the above, is "Extrait d'une mémoire sur la situation de l'hospice des insensés de la ville d'Avignon" (20 thermidor Year V): 270–73.

[159] See Roger Hahn, *The Anatomy of a Scientific Institution: The Paris Academy of Sciences, 1666–1803* (Berkeley and Los Angeles: University of California Press, 1971), pp. 217–18.

treatment was honored in this forum thus underscores its popular origins.

According to Broutet, a combination of *remèdes moraux* and *remèdes physiques* had been used at Avignon for almost twenty years[160] under the inspiration of English practitioners, especially Willis. Broutet gave a laudably precise account of the "moral" aspect, which he said consisted of "particular" and "general" strategies. The former, which were preferable, could be used when the exact cause of the insanity had been discerned, usually from information supplied by relatives. The "particular" strategies then aimed at "destroying" this cause by persuading the patient of its "imminent cessation." The example offered by Broutet was of a twenty-three-year-old woman whose father had volunteered that the cause of her madness was her jealousy of her brother; pressed by Broutet, the father acknowledged that this jealousy had an entirely real basis, for he had confiscated all of his daughter's earnings and used them to buy gifts for her male sibling. At Broutet's urging, the brother was now sent to another town for advanced training in his craft, and the father demonstrated his affection for the young woman by caresses and gifts. This constituted the *remède moral particulier*, which combined with physical means (baths, showers, purgatives, etc.) and general moral means produced a complete cure in something more than a month. The general moral means were "the most gentle (again, *douce*) consolations and a variety of distractions"–promenades, communal meals, games, pleasurable reading, music–skillfully combined and "lavished upon the patients."

Like Mourre, Broutet specified that the moral treatment be administered by laymen. Dr. Gastaldy (who will figure later in our story) had been employed at the Avignon hospice before the Revolution, but Broutet noted that his function–and that of the other accredited medical men who succeeded him–was to visit patients regularly and follow the course of each malady "in order to prescribe the most appropriate *physical* remedies." Elsewhere in the memoir Broutet was even more explicit and emphatic: "The administrators . . . to whom the lunatics would be entrusted [in his proposed scheme] would not have to take the trouble of becoming health officers (*officiers de santé*) as far as moral remedies are concerned." Broutet's proposal was that the pattern of the Avignon hospice be generalized, that the French government form comparable establishments, exclusively for lunatics and numbering one for every three departments. Broutet expressed suspicion of privately owned *pensions* for lunatics whose proprietors might be motivated by cupidity to skimp on the care of their patients. In keeping with this

[160] The medal elicited some debate over Avignon's priority and uniqueness in the implementation of the moral treatment in France. See the comments of the president of the Lycée des Arts in *Mémoire qui a obtenu une médaille*, p. 3 and p. 3 n9, and Pussin's testy complaint in his 1798 memoir, in Weiner, "Apprenticeship of Pinel," p. 132.

attitude, he even required that the administrators, who were to give the moral treatment in the public hospices he envisioned, be unsalaried, that they be chosen from among "affluent and charitable citizens," and that they dispense care "every day to this portion of suffering humanity . . . in an entirely disinterested manner," finding recompense only in the gratitude of their patients and their own "utility to the Republic." To be a provider of the moral treatment was, then, to have a self-abnegating secular calling, a republican vocation.

The final and most successful example of a non-Pinellian initiative to put the moral treatment into effect was the Revolutionary establishment of the Maison nationale de Charenton. Under the Old Regime the same buildings had housed the *pensionnat* of the Brothers of Saint-Jean-de-Dieu, which received lunatics on a charitable basis or for a fee and was also regularly used by the King as a prison for those seized by lettre de cachet. Tainted by this latter function, and on shaky financial grounds anyway after the state confiscation of monastic lands, the institution was closed by the Jacobins in the Year III.[161] Some two years later, however, the Executive of the Directory reopened it in fully secularized form, decreeing that "the hospice of the commune of Charenton, near Paris, known as a refuge for madmen, shall be restored to its initial purpose," and furthermore that it be turned into a model institution of its kind, a national showcase. "All necessary arrangements shall be made there to establish a *complete treatment* for the cure of insanity," for "there exists in France no establishment where madness is treated methodically by the medical art and in such a way as to arrive at a more verified cure." The new Charenton was intended to replace the lunatic wards of the Hôtel-Dieu, whose therapeutic techniques, once touted by Tenon, were now implicitly being labeled old-fashioned and inadequate. As soon as the "complete treatment" was put into effect at Charenton, the treatment of madness was to cease at the "Grand Hospice d'Humanité," as the Hôtel-Dieu was then called.[162]

Left vague in the original decree, the "complete treatment" was spelled out in the Year VI in a circular of the minister of the interior notifying the administrators of the departments of the reopening of Charenton. "Citizens, physical remedies are not the only ones to which we should resort for the cure of this malady; *moyens moraux* can also procure great successes. The doctor must call philosophy and the most tender humaneness to his aid." Hence the circular announced the appointment to Charenton of a *médecin philosophe*, one "who appears to have the greatest claim to this post." The man so honored had figured

[161] Imbert, *Le droit hospitalier*, p. 119; and the unsigned ms. "Notice historique et statistique sur la maison royale de Charenton" in AN: AJ^2 87, dated (on AN dossier) 6 août 1818. The *maison* was founded in 1641 and was taken over by the Order of Saint-Jean-de-Dieu in 1644.

[162] AN: AJ^2 60, decree of 27 prairial Year V, Preamble, Articles 1–2, my italics.

peripherally in the memoir for which Broutet had just received the medal of the Lycée des Arts. He was "Citizen Gastaldy, chief doctor at the hospice for the insane at Avignon since 1762," where he had contributed to the great fame of that hospice as a place where successful results were obtained by the "union of *secours moraux* and *remédes physiques*" and where the French had thus demonstrated that they were in no way inferior to the haughty English in such techniques.[163] A description of Gastaldy at Charenton written by one of his colleagues endows him with all the "classic" attributes of the practitioner of the moral treatment. "His gentleness (*douceur*), amiability, and general good nature in his relationships with the patients endeared him to these unfortunates, who considered him their father and comforter. Giving voice to his philanthropic musings, he used to say to me, 'A quarter of an hour of gaiety that we procure for the patients will, in effect, double our salaries and will be our sweetest recompense.' "[164]

"The success of the establishment," the ministerial circular read, "depends especially on the physician." But this statement did not anticipate the balance of power that would in fact evolve at Charenton, a balance of power far more consonant with the lay origins of the moral treatment and the never-implemented proposals of Mourre and Broutet. The story of the early years of Charenton thus reveals not only the affinity between the Revolution and the moral treatment but also the deep tensions produced by the ambiguous nature of the latter. The story deserves to be told here, at least briefly, because it clarifies the meaning of Pinel's historical role in laying claim to the moral treatment for official medicine and launching the specialty of psychiatry by effecting the transformation of charlatanism.

The dominant figure at the newly refounded Charenton was not the physician Gastaldy but the director, François-Simonnet de Coulmiers. This latter, also appointed by the government, had (as he put it with characteristic self-assertiveness) "taken possession" of his post in the Year VI.[165] Formerly a cleric, a member of the regular order of canons, the Prémontrés, de Coulmiers had gone under the name of the Abbé d'Abbécourt during the Old Regime and the early days of the Revolution. He was elected to the Estates-General in 1789 as a deputy of the first estate from Paris and soon emerged as a highly vocal member of the liberal clergy, speaking out in favor of the meeting together of the three estates in a national assembly and of the unerring rectitude of majority rule, and later urging his fellow clerics to swear the civic oath

[163] Ibid., circular of 1 nivôse Year VI.

[164] AN: AJ^2 100, "Précis sur la maison de santé du gouvernement à Charenton," dated September 1812, ms. pp. 8–9. Unsigned, but from textual evidence clearly by de Coulmiers.

[165] Ibid., ms. p. 3.

in ratification of the Civil Constitution of the Clergy.[166] Unlike the Abbé Sieyès, another and more famous liberal cleric, the Abbé d'Abbécourt did not abandon his clerical identity in 1789 but championed the Revolution precisely from that vantage point, taking upon himself the role of shaping clerical opinion in a revolutionary direction. In February 1790 he was appointed to the Comité de mendicité as one of its four original members. There he met Thouret,[167] a circumstance which no doubts explains how he became part of that medical-bureaucratic nexus from which his appointment at Charenton eventually issued.

At Charenton, the moral treatment became as much the province of the lay concierge de Coulmiers as of the medical man Gastaldy. De Coulmiers described Gastaldy as "my counsel and my friend"; "we sought *together*," he wrote, "the means of dispelling [delirious ideas] by innocent games, concerts, dance, comedies in which the roles were played by the patients"–and this set of jointly devised therapeutic techniques, having distraction as their unifying principle, de Coulmiers called "this moral treatment."[168] Nor was Gastaldy a constant presence at the institution. Already of an advanced age when appointed, he made his medical visits there only every other day, and during the winter, "given the rigor of the season and the condition of the roads," often only once every four days.[169]

When Gastaldy died in 1805, it was de Coulmiers's firm intention to continue the quasi-lay regimen that had developed at Charenton since its refounding. His candidate for Gastaldy's successor was, logically enough, an insider–the surgeon at the institution, who "had accompanied Gastaldy on his visits for eight years" and had acquired "the habit of treating insanity" from this unpresumptuous man.[170] But de Coulmiers's preference was ignored in favor of a protégé of a divisional chief of the Ministry of the Interior. Suspicious from the outset of this candidate, de Coulmiers claimed that he had sought the opinion of his old colleague Thouret, who was reportedly "a little equivocal; he said nothing too affirmative about the talents of the subject."[171] But it is clear that Thouret and the other representatives of established medi-

166 See "Motion de M. de Coulmier, abbé d'Abbécourt, député du clergé de Paris, extra muros," n.d., BN: Le[29] 2142, esp. p. 1 n1; and "Opinion sur le serment civique par M. l'abbé d'Abbécourt, député à l'Assemblée nationale," 1791, BN: Ld[4] 3229.

167 C. Bloch and A. Tuetey, eds., *Procès-verbaux et rapports du Comité de mendicité de la Constituante, 1790–91* (Paris: Impr. nationale, 1911), pp. 1, 22.

168 "Précis sur la maison de santé," ms. pp. 9–10, my italics. This therapeutic tenet was also de Coulmiers's justification for granting permission to a famous inmate, the Marquis de Sade, to stage plays at the asylum, the act for which de Coulmiers is best remembered today.

169 AN: AJ[2] 100, Draft of "Rapport au Ministre de l'Intérieur sur la nomination d'un médecin," n.d., from textual evidence written by de Coulmiers.

170 Ibid.

171 "Précis sur la maison de santé," ms. p. 11.

cine in the capital in fact intervened on behalf of this candidate in order to prevent virtually complete lay control of Charenton.[172] The new chief physician of Charenton was Antoine-Athanase Royer-Collard, who had received his medical degree in Paris three years before and who had a vastly different conception of medical power and its exclusivity than had the relaxed and latitudinarian provincial Gastaldy. Royer-Collard's insistence upon the superior authority of the physician in an institution for the insane provoked years of bitter quarrels with the equally strong-willed de Coulmiers.[173]

But while this lay–medical battle continued to smolder inconclusively at Charenton, its ultimate outcome had in a sense already been decided within the upper reaches of the bureaucracy. For as we have seen, the insane of the Hôtel-Dieu were to have been transferred to Charenton (as soon as the "complete treatment" had been installed there) in recognition of Charenton's status as the successor to the Hôtel-Dieu, the new-style "model institution" of its type in France. Not only were the lunatic wards of the Hôtel-Dieu to have been emptied "from that day forward" but, the 1797 decree of the minister of the interior continued, "the disease of madness shall no longer be treated *in any other Paris hospice*."[174] In 1802, however, the Ministry of the Interior substantially reneged on this promise, sending only a portion of the Hôtel-Dieu population to Charenton and a large number to the Salpêtrière, the institution under Pinel's medical aegis.[175] The official, statist choice for the medicalized moral treatment of Pinel over the lay moral treatment of de Coulmiers had been made.

[172] This is the opinion of Esquirol in his highly polemical comments in "Mémoire historique et statistique sur la maison royale de Charenton," *Ann. d'hyg. pub.*, 1st ser., 13 (1835): 45. His opinion is confirmed by a much less biased source; see AN: AJ[16] 6308, "Séance publique de l'Ecole de médecine de Paris du 10 novembre 1806, Discours de M. le Professeur Jussieu," p. 22.

[173] See AN: AJ[2] 100, dossier labeled by the archivist "Démêlés de Royer-Collard et de Coulmiers." A typical reprimand of the overweening de Coulmiers by the irate Royer-Collard: "It is to me and not to you that the patients are entrusted for everything concerning their malady and its treatment. . . . As for you, you are charged with furnishing [them] with food, bedding, laundry and blankets, with keeping them clean, with insuring that all the measures . . . prescribed by the chief physician are executed." (Draft of a letter dated July 9 1813.) De Coulmiers's warfare included informing the minister of the interior, in the wake of the Mallet du Pan conspiracy, of Royer-Collard's lack of Bonapartist sentiment; see his letter dated November 22, 1812. The attitude of official medicine toward de Coulmiers's role at Charenton is also expressed, retrospectively and acerbically, by Esquirol in "Mémoire historique et statistique," pp. 44–45. I have analyzed another aspect of the quarrels between de Coulmiers and Royer-Collard in "Foucault among the Sociologists: The 'Disciplines' and the History of the Professions," *History and Theory* 23 (1984): esp. 188–91.

[174] Decree of 27 prairial Year V, Article 2, my italics.

[175] Marcel Gauchet and Gladys Swain give the details of the 1802 transfer in *La pratique de l'esprit: L'institution asilaire et la révolution démocratique* (Paris: Gallimard, 1980), pp. 53–54.

Healthy sentimentality

If the political resonance of the moral treatment helped it to achieve paradigmatic status, so too did its cultural resonance. Advocates of the moral treatment can be seen as proto-Romantics, declaring that the intellectual life did not embrace the whole human being, that the affective life had also to be considered. This was Pinel's essential point when he first noted the insufficiency of Condillac's philosophy and the corresponding impropriety of defining insanity solely in terms of intellectual errors, without regard to passional aberrations. Pinel's anti-intellectualist bias became more pronounced with time. For example, the brief discussion of the passions in the introduction to the first edition of the *Traité* – largely a precis of Crichton – gave way, by the second edition, to a new, full-blown typology of the passions as causes of insanity.[176]

As a result of this passional emphasis, descriptions of the moral treatment could take on a tone of effusive sentimentality that even verged on mawkishness. Thus, in the case of the guilt-ridden tailor, Pinel observed that, after the patient was set to work repairing garments and had emerged from the blackest depths of his melancholy, "he even spoke with a tender concern of his six-year-old child, whom he earlier appeared to have forgotten, and evinced an intense desire to have the boy near. This revival of his *sensibilité* appeared to me to be a most happy augury." The cycle was repeated: When melancholy again engulfed the tailor, he resumed his "indifference or rather aversion to his child, whom he seemed to push aside with disdain"; but after the use of the mock trial restored his mental equilibrium, "he solicited with all the expression of *sensibilité* the return of his child." The young Esquirol generalized this point about the annihilation of the natural *sensibilité* and its revivification through the moral treatment. "A symptom which accompanies all insanities – and which has not escaped Professor Pinel, who has given notice of it numerous times – is the alteration of the moral affections. Sometimes the attack is ushered in by coldness and aversion towards close relatives and intimate friends; sometimes the dominant idea of the insanity is that being in the bosom of one's family is intolerable. . . . This moral perversion is so striking, so constant, that is appears to me to be an essential characteristic of insanity . . . and [conversely] the desire to see relatives and friends, the joy, the tears of *sensibilité* in seeing them . . . offers an infallible sign of an imminent cure."[177] In Moreau de la Sarthe's case histories, where moral means are used to alleviate the emotional disorders of

[176] *Traité*, 2d ed., pp. 25–39. See also Moreau de la Sarthe's review of Pinel, *Traité*, 2d ed., in *Moniteur universel*, August 17, 1811, p. 877.

[177] Pinel, *Traité*, 1st ed., pp. 234–37; Esquirol, *Des passions*, p. 29.

soldiers wounded in the Napoleonic wars, healing power is imputed to a full, cathartic expression of painful feelings. An amputee "abandons himself without reserve and confides all his fears, his harrowing anxieties" about the future of his family to the *homme sensible* who has questioned him; the latter "cannot hold back his tears and lets them fall on the sickbed." Another soldier, filled with nostalgia for his home department, is encouraged to talk about that beloved place. "Several conversations, some moving scenes, [and finally] abundant tears that, until then, a spasm of repressed (*concentrée*) sadness had hardly allowed to flow."[178]

Thus certain stereotypic marks – tenderness for little children, the blissful harmony ascribed to the family circle, tears flowing freely and copiously in response to familial joys or sorrows – connect the moral treatment and its proponents' concept of human nature to the eighteenth-century cult of sentimentality, or, as it is sometimes called, the preromantic mode. It is no wonder then that Moreau de la Sarthe named Samuel Richardson's *Sir Charles Grandison* as a novelistic source for the dissemination of the idea of the moral treatment: Richardson was one of the chief representatives of the cult of sentimentality and was hailed in France for his ability "to make the passions speak."[179] Similarly, Pinel's Rousseauism takes on an added dimension when seen from this vantage point. The author whose *Nouvelle Héloïse* had called forth floods of "delicious" tears from his grateful readers[180] would have special significance for the physician who regarded an appropriately tearful sentimentality as a definitive sign of mental health.

Pinel was concerned to present the moral treatment as a lay-inspired and nonetheless thoroughly scientific approach to the cure of madness. But without in any way abandoning the framework of his scientific

[178] Moreau de la Sarthe, "Encore des réflexions et des observations relatives à l'influence du moral sur le physique, et à l'emploi médical des passions, des affections et des émotions," *Décade philosophique* (20 nivôse, 30 nivôse Year IX): esp. 74–75, 134–37. Moreau's use of the expression of pent-up feelings as a mode of moral therapeutics is quite different from Pinel's mode of "schooling" the passions; it also anticipates Freud's psychoanalytic technique of catharsis or abreaction.

[179] Diderot, "Eloge de Richardson" (1761), in *Oeuvres complètes*, Vol. 5, p. 215. Clementina in *Grandison*, on whom some variant of the moral treatment was used, was a particularly "sentimental" character and gained a wide following on the Continent for this reason; see Margaret Anne Doody, *A Natural Passion: A Study of the Novels of Samuel Richardson* (Oxford: Clarendon Press, 1974), pp. 326–27; and Diderot, "Eloge de Richardson," p. 226.

[180] See Robert Darnton, "Readers Respond to Rousseau: The Fabrication of Romantic Sensitivity," in *The Great Cat Massacre and Other Episodes in French Cultural History* (New York: Basic Books, 1984), esp. pp. 242–48, describing and quoting from the "fan mail" that Rousseau received in the 1760s.

rationalism, he wished to recognize the claims of sentiment as well. His was an *âme sensible* conjoined with an *esprit éclairé*,[181] and his moral treatment was entirely suited to the general cultural trend of his day, to Romanticism emerging out of Enlightenment.

[181] See Georges Gusdorf, *Naissance de la conscience romantique au siècle des lumières* (Paris: Payot, 1976), title of Ch. 2 of Part 1.